

27 April 2023

Police Interviews - Baby Q & general matters

Eirian Powell

Summary of agreed facts including police interviews, exhibits & staff presence

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Thursday, 27 April 2023

(10.30 am)

(In the absence of the jury)

Housekeeping

MR JUSTICE GOSS: Before the jury comes in, I've received a note about days that a juror has hospital appointments. I won't comment on the fact that yesterday, which was a hospital appointment, didn't actually take place and has now been rescheduled, but he only found out when he got to hospital. I say no more.

So 4 May and 10 May are days to be added that we won't be able to sit with the jury, which means that next week the only days we will be sitting will be Tuesday and Friday.

MR MYERS: My Lord, yes.

MR JUSTICE GOSS: I received your helpful note, Mr Myers.

Perhaps we could come to that later today.

MR MYERS: Yes.

MR JUSTICE GOSS: Then the following week, which begins with the public holiday on Monday the 8th, we will not be sitting the 8th, 9th or 10th, so we'll only be sitting 2 days of that week.

Then we have, we hope, a clear run through to Monday the 22nd.

MR MYERS: That's where we are, my Lord. It's impossible to --

MR JUSTICE GOSS: Exactly. It's not this juror's fault at all. It's just a combination of unfortunate circumstances.

MR MYERS: And a number of holidays that fall as well.

MR JUSTICE GOSS: Well, exactly, three public holidays in a month. Right. So there we are.

Mr Johnson, I gather that you will have to leave early this afternoon.

MR JOHNSON: It's not this afternoon, it's 10 May.

MR JUSTICE GOSS: Oh right. Sorry, I was told it was this afternoon. Well, that's all right. That's at least one happy coincidence. We'll carry on then now.

I should say also before the jury comes in, you have seen my ruling in relation to that.

MR MYERS: Yes, we're grateful, my Lord.

MR JUSTICE GOSS: In relation to all except the last one -- I think it's fairly clear what it is. The last one, I'm just hoping that between you, you can agree the way that can be done rather than --

MR MYERS: I should say it's something I was going to steer round, that aspect.

MR JUSTICE GOSS: You can do. But as long as it's done perhaps in agreed leading questions. That might be the best way.

MR MYERS: Or it may be that I leave it to some other way as we go along. If they're dates that are not contested in some way, there's a variety of ways of putting that before the jury without getting into it with the next witness.

MR JUSTICE GOSS: Thank you very much. Sorry, I should have mentioned that as well. I got sidetracked by the disappointing news.

MR MYERS: Me too.

MR JUSTICE GOSS: Thank you.

(In the presence of the jury)

MR JUSTICE GOSS: Good morning, members of the jury. I'm sorry that you've been kept waiting for a few minutes.

I have been discussing the note that one of you has sent me, which I have now read and discussed with counsel.

The consequence is that I am adding to the days we are not going to be sitting, unfortunately. It's just a very unfortunate combination of circumstances but there we are. So I will, before we part company at the end of today, give you a revised list of non-sitting days, adding two more days. It's going to mean that in fact from next week we're going to have two two-day weeks, but there we are. We'll carry on now. I'll come back to that later today.

MR ASTBURY: My Lord, we'd reached the first of the interviews concerning [Baby Q], which is very much towards the end of bundle 2.

DS DANIELLE STONIER (continued)

Examination-in-chief by MR ASTBURY (continued)

MR ASTBURY: The first of these interviews regarding [Baby Q] took place on 5 July 2018.

STONIER: Yes, that's correct.

ASTBURY: It began with the usual introductions and the officer began: Okay, so during this interview we're going to talk about [Baby Q]. Okay? At 04.09 hours on 22/6/2016 [Baby Q] was born to [Mother of Baby Q] and [Father of Baby Q]. At 09.10 hours on 25/6/16, [Baby Q] collapsed. Okay? What can you tell us about the care that you offered to [Baby Q], Lucy?

STONIER: So I believe on that morning I was looking after [Baby Q] and another baby that was in nursery 1. [Baby Q] was in nursery 2 and from what I remember I was alerted by another member of staff that he'd vomited and needed some support when I was in the other nursery. That's all I can remember from memory.

ASTBURY: Lucy Letby confirmed that she had the relevant notes and that she had been [Baby Q]'s designated nurse on the relevant shift. She was asked: Okay. And, just generally, what can you tell us about the clinical position of [Baby Q] when you took over his care on that morning?

STONIER: Um, [Baby Q] had a low temperature, which had needed his incubator to be increased and also that he was tachycardic.

ASTBURY: At the time you took over?

STONIER: Yes.

ASTBURY: Okay. So what was being done about that?

STONIER: So his be -- his incubator was being increased to combat the temperature.

ASTBURY: Okay. So in terms of what you needed to do with him from the beginning of the shift, moving forward with his care plan, what -- what was in your mind that you were going to progress with that?

STONIER: With his care for the day?

ASTBURY: Yes.

STONIER: I would be reviewing what medications he was due and when, how often he was needing observations, if and when he was being fed and when they were due, when his nappy change would be due.

ASTBURY: Okay. Was he ventilated?

STONIER: No.

ASTBURY: Lucy Letby was unable to recall which other baby was in nursery 2 at the time, but she did remember that Mary Griffith was the other nurse working in there. She was asked: Okay. So with regards to his position when you took over that morning, were there any concerns for him at that time?

STONIER: That he had a low temperature.

ASTBURY: Okay. Sorry, just on that point, were you concerned such that you were happy to leave [Baby Q]?

STONIER: Leave in what way?

ASTBURY: Well, you said that -- at the initial start of the interview you gave you said you were alerted to the fact that he'd vomited [as read].

STONIER: Yes.

ASTBURY: So where were you?

STONIER: I believe I was in nursery 1 with the other baby.

ASTBURY: Right, okay. So the point I'm saying is [Baby Q] was well enough to be left?

STONIER: Yes.

ASTBURY: Yes. Okay. The next entry is at 9.10. Explain that entry to us.

STONIER: Okay. So I have written that he was attended to by Staff Nurse Lappalainen, who I think was in charge that day.

ASTBURY: Okay.

STONIER: I think I'd been in nursery 1 and I came back out to come to nursery 2 and saw that [Baby Q] was having intervention and that I've written, from here, he had vomited, mottled, desaturation in (sic) and had needed Neopuff and suction and that [Dr A] had attended.

ASTBURY: Okay so at 09.10 when you handed him over, why was that?

STONIER: I haven't handed him over. This -- Staff Nurse Lappalainen had attended to [Baby Q], so she had gone to him for a reason.

ASTBURY: Because you were out in the other nursery?

STONIER: Yes.

ASTBURY: Do you know what alerted her to go in and attend to him?

STONIER: No.

ASTBURY: Okay. And you were elsewhere dealing with another baby?

STONIER: I believe so, yes.

ASTBURY: Between 08.00 and 09.00 hours Lucy Letby explained that she'd completed [Baby Q]'s observations, he'd had a slightly high respiratory rate, and she increased the temperature of his incubator. And she was asked: Would that -- is that something that needs any kind of treatment, the increased respiratory rate?

STONIER: No, so usually, you just find -- usually if they enter this other band in here on the chart --

ASTBURY: Right, okay.

STONIER: -- then we would -- you would let somebody know or the doctors would review them when they were next on the unit.

ASTBURY: Okay.

STONIER: So the doctors usually attend to do the ward round at 9 o'clock. I don't think I escalated that at that point.

ASTBURY: Okay. Can you remember any other treatment around the observations just prior to your leaving?

STONIER: No.

ASTBURY: Lucy Letby confirmed from the notes that she'd not fed [Baby Q] and he was receiving Babiven and lipids.

Question: Did you communicate with any other members of staff that you were leaving the nursery?

STONIER: I think Mary Griffith was in the room when I left and I think I told her.

ASTBURY: Okay. Do you remember when you told -- you think you told Mary what -- what might Mary have been doing at the time?

STONIER: I think she was with another baby in the nursery.

ASTBURY: Was she doing anything to that baby, treating that baby at all?

STONIER: I think she was at the incubator, yes, from what I remember.

ASTBURY: Okay. When -- do you remember the words you said to her?

STONIER: No.

ASTBURY: Okay. So how did you first become aware that [Baby Q] needed to be attended to?

STONIER: I think from memory I came out of whatever I was doing in nursery 1 and saw that people with [Baby Q] in nursery 2.

ASTBURY: What were they doing?

STONIER: I think when I went in they were giving him support with the Neopuff.

ASTBURY: Okay. And who was that?

STONIER: I think it was Minna Lappalainen.

ASTBURY: Just on her own?

STONIER: No, I think -- I don't know if a doctor was there or Mary was there as well.

ASTBURY: Okay. So what did you do then? What observations did you make?

STONIER: I don't remember entirely but I believe he was moved into nursery 1 and we started CPAP on him.

ASTBURY: Okay. Did you see the vomit?

STONIER: I don't remember the vomit.

ASTBURY: Okay. So where put he'd:

"... vomited clear fluid nasally and from mouth, desaturation, bradycardia, mottled ++."

Were they your observations or were they what you were told?

STONIER: I believe they were what I was told.

ASTBURY: Okay. And the Neopuff and suction applied, are they actions that you've done or actions that people who were treating him have done?

STONIER: No, I think they were actions by other people.

ASTBURY: Okay. So did you -- I don't know if I asked you, sorry, did you see the vomit?

STONIER: No, I don't think I saw the vomit.

ASTBURY: But that's how it was described to you?

STONIER: Yes.

ASTBURY: And so after you were told the description of what some of your colleagues have seen what did you think about what had happened to [Baby Q]?

STONIER: I was unsure as to why he would have been vomiting.

ASTBURY: Okay. What are the implications of a clear fluid, nasally and from the mouth?

STONIER: That he's vomited, but he hasn't got anything in his stomach to vomit, so it's clearly bodily fluids that he's vomiting.

ASTBURY: So when you left -- did you do a procedure for [Baby Q] then prior to leaving?

STONIER: I've documented observations at 9 o'clock.

ASTBURY: Okay. Did you administer anything to [Baby Q] before you left?

STONIER: I don't know from memory I'd have to check.

ASTBURY: If you could that would be great.

STONIER: No, I don't think so.

ASTBURY: The observations at 9 o'clock, how long does that process take?

STONIER: Not long because we read them from the monitor, then we count the respiratory rate.

ASTBURY: Are we talking seconds, minutes?

STONIER: A minute maybe.

ASTBURY: I presume if you were concerned you -- would you stay with [Baby Q]?

STONIER: Yes.

ASTBURY: Did you see the mottling on [Baby Q]?

STONIER: I don't remember.

ASTBURY: Right, okay. And was this vomit and desaturation -- was that expected from your point of view, from what you'd observed from [Baby Q]?

STONIER: No, but sometimes babies do vomit and they can -- that can lead them to have a desaturation because they have vomited.

ASTBURY: Okay. So the next line is:

"[Dr A] attended. Air ++ aspirated from NG tube. Transferred to nursery 1."

Okay. So the air ++, are they your entries and your observations?

STONIER: I'm not sure whether I aspirated the tube or whether that was done by somebody else whilst I was there.

ASTBURY: Right okay. So air ++ from the NG tube, how does that happen?

STONIER: How do you get air in the --

ASTBURY: Yes.

STONIER: I am not sure. Sometimes if babies are vomiting they can gulp down air.

ASTBURY: Right. Are there any other ways that air can get there, air ++ especially?

STONIER: I'm not sure.

ASTBURY: Okay.

STONIER: If the baby's maybe got some sort of blockage in the bowel, that air isn't passing through the rectum.

ASTBURY: Any others?

STONIER: No, not that I know of.

ASTBURY: Who contacted the registrar?

STONIER: I'm not sure.

ASTBURY: Okay. Was he one of the people that were in attendance when you first became aware of members of staff treating [Baby Q]?

STONIER: I don't recall specifically who -- if he was there or not at that moment, no.

ASTBURY: Okay. And you don't recall whether you aspirated the tube?

STONIER: No.

ASTBURY: Okay. Did you continue to care for him after that?

STONIER: Um, I think so, yes.

ASTBURY: As designated nurse, I mean.

STONIER: Yes.

ASTBURY: Lucy Letby explained that [Baby Q]'s parents visited later that day. They were upset that nobody had told them about what had happened. She and [Nurse B] had apologised that it must have been an oversight. Whilst [Baby Q] was being treated as the priority, they had not had chance to contact the parents. And she was asked: Okay, are there any other observations you've got regarding [Baby Q]?

STONIER: No.

ASTBURY: Can you give us any explanation as to why this event happened, where he vomited and collapsed?

STONIER: No.

ASTBURY: And the interview, insofar as [Baby Q] was concerned, was suspended at that point.

STONIER: Yes.

ASTBURY: Thank you.

Moving on to the second occasion when Lucy Letby was asked questions about [Baby Q], we can see that took place on 12 June 2019.

STONIER: Yes, that's right.

ASTBURY: It began with the observation: Lucy, I'm going to ask you about [Baby Q].

The officers then summarised the previous interview concerning [Baby Q]. Ms Letby was told and then asked: Mary Griffith states Nurse Letby was also caring for a second baby in nursery 1. Which baby were you caring for in nursery 1?

STONIER: I think her name was B.

ASTBURY: That's our editing, the full name was given.

STONIER: Yes, it was, yes.

ASTBURY: Well, certainly the first name: You left the nursery shortly before [Baby Q] collapsed?

STONIER: Right.

ASTBURY: Do you agree with that?

STONIER: Yes.

ASTBURY: Lucy, explain to me what did to [Baby Q] before leaving nursery 2 to cause his collapse?

STONIER: I didn't cause his collapse, I checked his observations.

ASTBURY: Okay. And what did his observations suggest? Was he stable?

STONIER: That his temperature had decreased, so I've increased his incubator.

ASTBURY: Mm-hm. Was he stable at that time Lucy?

STONIER: He's got a rise in respiratory rate and heart rate but it's not going completely into the warning area so...

ASTBURY: Okay. Would you class [Baby Q] as stable at that time?

STONIER: Yes.

ASTBURY: Would you have left the nursery if he wasn't?

STONIER: No.

ASTBURY: What did you do?

STONIER: And I've asked Mary to keep an eye on him in my absence.

ASTBURY: What did you do to him, Lucy, to cause him to collapse?

STONIER: I took his observations, I didn't cause a collapse.

ASTBURY: There was two people in the room, Lucy, wasn't there, Mary Griffith and you?

STONIER: Yes.

ASTBURY: Mary was treating another baby and then [Baby Q] collapses. Are you responsible for his collapse?

STONIER: No.

ASTBURY: Lucy Letby was informed of Dr Evans' opinion regarding air via the NGT into his, that meaning [Baby Q]'s, stomach.

STONIER: I don't think I fed him at that point, did I? I think I just did observations.

ASTBURY: [Baby Q] suffered a single collapse; do you agree?

STONIER: Yes.

ASTBURY: Did you inject air into [Baby Q]?

STONIER: No.

ASTBURY: Were you responsible for his collapse?

STONIER: No.

ASTBURY: Lucy, are you responsible for the attempted murder of [Baby Q]?

STONIER: No.

ASTBURY: Okay. It's quarter to 12 and the interview is now at an end.

That was the conclusion of that particular interview.

STONIER: Yes.

ASTBURY: The third and final occasion when Lucy Letby was asked questions about [Baby Q] was on 11 November 2020.

STONIER: Yes.

ASTBURY: The interview begins: You were away from [Baby Q] when he suffered a desaturation and vomited. You denied causing [Baby Q] to collapse and denied causing him any harm. Is that accurate?

STONIER: Yes.

ASTBURY: Is there anything you want to add regarding that?

STONIER: No.

ASTBURY: At 09.00 hours you completed [Baby Q]'s observations and he was due a feed. However, this was not given. When Minna attends to [Baby Q] after his desaturation she evidences that he had quite a bit of mucus and he'd been sick. If it wasn't feed, what had you given to him to cause him to vomit?

STONIER: I didn't give him anything. If it's not documented that I didn't feed him then I didn't give him anything.

ASTBURY: Was it air that you administered?

STONIER: No.

ASTBURY: Did you deliberately leave the room to blame the collapse on Mary Griffith and Minna?

STONIER: No, the baby in nursery 1 was due cares at that time.

ASTBURY: Dr Lakin from Alder Hey Children's Hospital shows that he made a quick recovery. Do you agree with that?

STONIER: Yeah, that's actually what happened, yeah. That's not stay that he wouldn't have recovered if he had stayed with us on the unit.

ASTBURY: The officers summarised the statement of [Dr D], who describes [Baby Q] as having been stable overnight on the shift on the 24th into 25 June: Is that just a coincidence then, is it?

STONIER: Yeah. Babies can deteriorate any time.

ASTBURY: At 13.30 hours on the 25th you messaged [Nurse E] and said in speech marks, "Aspirated". Was he aspirated?

STONIER: I don't recall, but I think "aspirated" meant that when a baby's vomited and then inhaled the fluid back into their lungs, that's an aspiration.

ASTBURY: Do you remember doing that?

STONIER: Doing what?

ASTBURY: Aspirating.

STONIER: No, so aspirated would be me withdrawing the feed from him and aspiration -- if I'm saying that he's aspirated, it means he's been sick and then inhaled some of the fluid, which is like pneumonia.

ASTBURY: Okay.

STONIER: I am saying he's aspirated then, that's the context I think.

ASTBURY: At 22.46 hours on the 25th you messaged [Dr A] and asked if you should be worried about what Dr Gibbs was asking; do you recall that?

STONIER: Yes.

ASTBURY: What was that about then, Lucy?

STONIER: I became aware that Dr Gibbs had been asking why -- either why or where I was when [Baby Q] collapsed, why I wasn't in the nursery with him, and it was discussed that obviously I had two babies in separate nurseries and I was concerned that I was going to be -- you know, be a problem that I wasn't there at the time.

ASTBURY: Were you trying to seek his reassurance?

STONIER: Yeah, I suppose so, yeah. I wouldn't have just left a baby unattended: Mary was in the room and Minna was just outside at the desk.

ASTBURY: That wasn't trying to blame the others?

STONIER: No, there was no blame to be apportioned. It was just that I had not left the nursery unattended to tend to my other baby.

ASTBURY: At 16.44 on the 26th you messaged [Nurse E] and told her [Baby Q] had NEC and that [Dr A] had told you; is that correct?

STONIER: I don't recall that specifically, no.

ASTBURY: Did [Baby Q] have NEC?

STONIER: I'm not sure without looking.

ASTBURY: [Nurse E] tells you that [Baby Q] may have volvulus; do you remember that?

STONIER: No.

ASTBURY: Were you concerned people were talking about [Baby Q] and what was going on with him and why he deteriorated?

STONIER: I don't think I was worried. I think it happened because we were concerned why it had happened to him, yeah. And if [Nurse E] had -- I don't remember that, but if [Nurse E] had heard that he possibly had volvulus she would have wanted me to know that.

ASTBURY: What is a volvulus?

STONIER: I think it's like a twist in the bowel, in the abdomen.

I'm not 100% sure.

ASTBURY: Okay. Thank you.

Then: Okay. That's the end of the interview.

And the time is given as 11.06.

STONIER: Yes.

ASTBURY: I'm just asked to confirm that when Ms Letby initially confirmed which nursery she'd been in, that was a point before she had the notes before her; is that right?

I think it's in the first interview.

If we go to [document redacted] -- it's the second interview, in fact.

STONIER: Yes.

ASTBURY: At the bottom of [document redacted]:

"Which baby were you caring for in nursery 1?"

And then at the top of [document redacted] we have the name given.

STONIER: Yes.

ASTBURY: Do you recall whether Ms Letby had her notes in front of her at that time or was that from memory?

STONIER: I believe she would have had her notes with her in front of her at that time.

ASTBURY: Okay. Those notes presumably would have been the notes from [Baby Q], not the other baby?

STONIER: Yes, that's right.

ASTBURY: Thank you. Now, we move on to what's described as OA.

Now, the interviews didn't just centre on the babies themselves, although that took up most of the interviewing; is that right?

STONIER: Yes, that's right.

ASTBURY: There were more general questions about, for example, exhibits found at Ms Letby's home?

STONIER: Yes.

ASTBURY: And also certain general events at the hospital and practices; is that right?

STONIER: That's correct.

ASTBURY: The next four interviews again have been distilled into that sort of topic and where babies were mentioned that's been taken out and put in their individual interviews; is that right?

STONIER: Yes.

ASTBURY: And these are described as overarching interviews, hence the OA?

STONIER: Yes.

ASTBURY: And there are four in total?

STONIER: Yes, there are.

ASTBURY: Thank you. So the first of those that we're going to look at took place on 3 July 2018; is that correct?

STONIER: Yes, that's correct.

ASTBURY: It took place in the evening. Just to remind everybody and to put it into context, that was the very day that Ms Letby was arrested for the first time?

STONIER: Yes, that's right.

ASTBURY: Okay. So introductions and caution repeated again.

It's a similar situation there are a number of interviews but they've been reduced to just the relevant parts; is that right?

STONIER: Yes.

ASTBURY: Lucy Letby was asked to tell the police about the note that she'd written with the exhibit reference NAC10; is that correct?

STONIER: Yes, that's correct.

ASTBURY: Again just to remind people that's the small Post-it that was inside the diary in the chest of drawers?

STONIER: Yes.

ASTBURY: Can you give the answer, please?

STONIER: I just wrote it because everything had got on top of me.

It's when I'd not long found out I'd been removed from the unit and they were telling me that my practice might be wrong, that I needed to read all my competencies, my practice might not have been good enough, so I -- I felt like people were blaming my practice, that I have hurt them without knowing through my practice and that made me feel guilty and I just felt really isolated. They made -- they stopped me speaking to people and...

ASTBURY: Do you want to elaborate on some of the things that you've put down in there?

STONIER: I was blaming myself, but not because I'd done something, because of the way people were making me feel, but like -- I'd only ever done my best for these babies and then people were trying -- trying to say that my practice wasn't good and that I'd done something and I just couldn't cope. And I just didn't want to be here any more.

ASTBURY: Do you remember what you wrote down?

STONIER: I think I do.

ASTBURY: And then Ms Letby's solicitor interjected: I have read it out to her.

And she was asked: Yeah, would it help if we go through it then, Lucy?

STONIER: Yes.

ASTBURY: So at the top of the note -- I think you have seen a copy of the note, like you said.

The solicitor confirmed that. Then the officer quotes:

"Not good enough", you've written and underlined.

So my colleague is just putting it there in front of you.

STONIER: Because I felt like I was good enough, that people were trying to suggest that, that I hadn't been good enough for them.

ASTBURY: Which people were they?

STONIER: The trust and the staff on the unit.

ASTBURY: What sort of things were they saying?

STONIER: Just that I'd been there for a lot of the deaths and they were going to review all my competencies because at that point they didn't know -- hadn't a clue what had happened and they wanted me to redo all my competencies in case there was something wrong with my practice and competencies.

ASTBURY: You go on to say, "There are no words, I can't breathe, I can't focus". Do you want to go through what was going through your mind at that time?

STONIER: I just felt it was -- it was all just spiralling out of control, I just didn't know how to feel about it or halves going to happen or what to do.

ASTBURY: When was this written?

STONIER: I think it was the July time, after I'd been removed from the unit.

ASTBURY: So July 2016?

STONIER: Sorry, yeah, 2016.

ASTBURY: And then you go on to say, "Kill myself right now, overwhelming fear and panic". Do you want to describe how you're feeling there?

STONIER: Pardon?

ASTBURY: Can you describe to me how you were feeling there?

STONIER: As I put there, it just felt that it was all -- it was all happening out of my control.

ASTBURY: Did you talk to anyone about that?

STONIER: I went to the GP.

ASTBURY: Your own GP?

STONIER: Yes.

ASTBURY: Did you get any help?

STONIER: Yes, just some antidepressants.

ASTBURY: When you say, Lucy, that the trust said they were going to review your competencies, can you be more specific with that?

STONIER: So when I was removed from the unit, it happened in July, and I met with the head of nursing and they told me that there'd been a lot more deaths and that I'd been linked as somebody that was there for a lot of them and they also said that there were some other people that had been flagged as being on shift for a lot of them and myself and these other people are going to have to be going and redoing our competencies.

ASTBURY: What do you mean by competencies?

STONIER: So competencies, to do things on the unit. So equipment competencies and transfusion competencies. We have competencies for most things, clinical care that we give on the unit.

ASTBURY: And who assesses those competencies?

STONIER: The practice education development nurse on the unit.

ASTBURY: Right, okay. So who were those other people?

STONIER: I was never told who.

ASTBURY: Right, okay.

STONIER: I was just told that it -- the process wasn't happening just for me, it'd be happening for a number of people.

ASTBURY: What do you think was going on with your competencies up to that point? Were you okay?

STONIER: Yes.

ASTBURY: Did you feel confident?

STONIER: Yes.

ASTBURY: Okay. So then on the back of that, did you have any concerns that there was a rise in the mortality rate?

STONIER: Yes.

ASTBURY: Okay, so tell me about that. What concerns did you have?

STONIER: I think we'd all just noticed as a -- as a team in general, the nursing staff, that this was a rise compared to previous years, um, and that we were meeting babies that had a lot more complex needs that we -- we weren't seeing a few years ago and it was talked about that this was something that was unusual.

ASTBURY: Okay, and what happened when that was recognised?

STONIER: Well, I believe things happened behind the scenes with management and the nursing team and they just carried on and just supported each other --

ASTBURY: Okay.

STONIER: -- and carried on as a team.

ASTBURY: At which point did it all become sort of the extent where you're saying things like, "kill myself now", "overwhelming"?

STONIER: It was when I was removed from the unit in the July of 2016.

ASTBURY: Right. Why at that stage did it culminate in those feelings?

STONIER: Because I suddenly felt that things had been directed towards me.

ASTBURY: Why was that?

STONIER: Because they were saying they were going to have to review my competencies, so I took it to mean my practice hadn't been good enough.

ASTBURY: Did you ever recognise that it wasn't? Did you ever make any mistakes?

STONIER: No.

ASTBURY: So in terms of "overwhelming fear and panic", what were you afraid of?

STONIER: That they were going to think that I'd done something wrong.

ASTBURY: Okay. And how would that -- what would happen if they thought that?

STONIER: If they thought that I'd done something wrong?

ASTBURY: Yeah.

STONIER: That this would happen --

ASTBURY: Right.

STONIER: -- that the police would get involved and I'd lose my job.

ASTBURY: And was it a job that you enjoyed?

STONIER: Yeah. Yeah, I loved my job.

ASTBURY: How does -- in your area, how does competencies or, you know, when people call into question your competencies, how does that lead to a police investigation?

STONIER: I don't know. I just panicked. I just thought if they found my competencies weren't good enough, it'd be assumed that I hadn't done -- like, missed something or not done something doing that I should have, that the babies had died or become unwell.

ASTBURY: Okay. How does that become a criminal matter though?

STONIER: I'm not sure. I thought they might refer me to the NMC and I didn't know if that went to the police.

MR JUSTICE GOSS: NMC?

MR ASTBURY: It's on the next page, my Lord.

The officer asks the same question: I don't know what that is.

STONIER: The Nursing and Midwifery Council who has our registration, who we are registered with. Just panic.

ASTBURY: What's the difference between being incompetent or somebody saying you're incompetent or criminal in your world?

STONIER: For the criminal it's something that's done deliberately, whereas you're not being competent would be that you're not competent in something that can give you a result that wasn't intentional.

ASTBURY: Okay. So in terms of where you say, "Kill myself right now", is that something that you were considering?

STONIER: Yes.

ASTBURY: Why was that?

STONIER: Because I just felt so isolated and alone and --

ASTBURY: Other than the doctor, did you speak to anyone else, family, friends?

STONIER: At the time I was because I was told I could only speak to two friends and I didn't want to tell them too much about it. The same with mum and dad, nobody knows.

ASTBURY: Did you get any support from work?

STONIER: They referred me to occupational health and things, yeah.

ASTBURY: You mentioned there that you were panicking. What were you panicking about?

STONIER: Just that it was all out of my control.

ASTBURY: So you were panicking about your -- personal emotions?

STONIER: Yes.

ASTBURY: In your own mind, had you done anything wrong at all?

STONIER: No, not intentionally, but I was worried that they would find that my practice hadn't been good.

ASTBURY: Are there any particular practices that you think might not have been as good as they should have been?

STONIER: No.

ASTBURY: What made you think that they might find something that was wrong or something that you shouldn't have done?

STONIER: It was more that I was worried that obviously they'd already gone to the lengths of redeploying me and moving me from the unit and banning contact. I didn't know how it was going to go. I didn't think that they'd find that I'd been incompetent, but I was worried that they might try and assume that I had been just because I was there for all these babies.

ASTBURY: Were you there for all those babies?

STONIER: Yes.

ASTBURY: In this note here you've written down:

"Police investigation forget."

What was going through your mind at that time?

STONIER: I was worried that the police might be involved.

ASTBURY: Like I said before, was there a reason why you thought?

STONIER: I think it was just panic at the time.

ASTBURY: Another word, "Slander, discrimination, victimisation".

STONIER: Because I felt that the trust and the team were trying to imply that it -- it was something that I had done.

ASTBURY: Was there any individuals that implied that?

STONIER: Yes, all the consultants.

ASTBURY: Go on, tell us who they are.

STONIER: Ravi Jayaram and Stephen Brearey.

ASTBURY: So what can you tell us about them?

STONIER: I just found out that they were the ones who had raised concerns about myself being the common factor in the deaths and that they felt that I'd deliberately harmed them.

ASTBURY: So do you want to tell us your professional relationship with Ravi Jayaram and Stephen Brearey? Did you have any issues with them?

STONIER: I felt we'd always had a good working relationship.

I've worked more with Ravi than Steve, but that was just through circumstances, who was on shift. But I always felt that we'd had a good working relationship.

ASTBURY: So do you think they -- can you give a reason why they might want to victimise you or point the finger towards you?

STONIER: It had crossed my mind at times whether they were trying to put the blame on me for something that somebody else had done.

ASTBURY: Are you aware of somebody else doing something?

STONIER: No.

ASTBURY: So when it crossed your mind what were you thinking?

STONIER: If they were questioning my competencies, that maybe they were questioning -- well, they told me they were questioning other people's as well or there'd been a competency issue with somebody else. They were trying to make it my problem because I was there.

ASTBURY: So up to that point that you say they might have discriminated against you or victimised you, you had no real issues with either Stephen Brearey or Ravi?

STONIER: No.

ASTBURY: No? No fallings out with them sort of professionally or?

STONIER: No.

ASTBURY: How did you get on with them personally?

STONIER: I didn't really know them in a personal capacity, only professional.

ASTBURY: Professional, okay. You go on to say in your notes: All getting too much, everything. Taking over my life. Everyone. I feel very alone and scared."

When you were writing these down, where were you, these notes?

STONIER: At home.

ASTBURY: Again, did you speak to anyone about this other than the doctor?

STONIER: No.

ASTBURY: Were you particularly close to anyone at work, Lucy?

STONIER: Yes.

ASTBURY: Who was that?

STONIER: My best friend is [Nurse E].

ASTBURY: Okay. Did you speak to her at all about how you felt?

STONIER: Not to the extent of wanting to kill myself, no.

ASTBURY: And then you put:

"How will things ever be like they?"

There on the sheet, what did you mean by that?

Then the officers added: And overwritten with "hate".

STONIER: How will things ever be like they used to.

ASTBURY: So what was going through your mind at that time?

STONIER: I'd been removed from the unit, I'd been banned contact with everyone, I couldn't see how it was going to go back to how it used to be.

ASTBURY: Why did you think that?

STONIER: Because the redeployment would go on my record, it would affect my practice, everything.

ASTBURY: So when you were redeployed, exactly what did they say to you when you were moved from the unit? Did they give an explanation?

STONIER: There'd been an increased mortality rate and that they needed to have an external review done. Until that was complete they wanted me to redo all my competencies.

And then it transpired they didn't have the staffing to facilitate that, so they redeployed me and said it would be on a temporary basis until the external review had been done and it was for my own protection.

ASTBURY: But you were thinking at this time thing aren't going to be the same again? But you were still employed up to this point as a nurse?

STONIER: Yes.

ASTBURY: Whereabouts was it?

STONIER: After? When I was redeployed?

ASTBURY: Yes.

STONIER: So the Risk and Patient Safety team in the Countess.

ASTBURY: What kind of department's that?

STONIER: It's a -- it's still part of the corporate nursing team.

They look at incidents and complaints and things that have come into the trust.

ASTBURY: Right.

STONIER: So I was moved into that department, office based.

ASTBURY: Lucy Letby then named the friends with whom she keeps in touch outside of work. And she was asked: Okay. When you said you were lonely, and if we sort of take out people from the Countess, that you didn't have a massive support network, is that how you felt?

STONIER: Yeah, yeah.

ASTBURY: Okay, so was that quite a big thing for you, leaving the unit and being told not to communicate with people?

Is that where the isolation --

STONIER: I'd lost everything, and obviously mum and dad were down in Hereford. And I thought we were a good team regardless who was my friends. We were a good nursing team on the unit and I'd just lost that. We were like a little family and I felt like I'd lost that.

ASTBURY: But what's the format of this? Obviously these are sort of emotional outpourings, would you say. How would you describe the thing as a whole?

STONIER: I think it was just a way of me getting my feelings out on to paper. It just helps me process it a bit more, I think.

ASTBURY: Okay. Is that all in one session, if you like?

STONIER: I believe so, yes.

ASTBURY: Is this how your emotions would manifest themselves, an outlet?

STONIER: Yes.

ASTBURY: Okay. Lucy, you then go on to say that:

"I don't deserve to live. I killed them on purpose because I'm not good enough to care for them. I am a horrible, evil person."

STONIER: I didn't kill them on purpose. I felt if my practice hadn't been right, then I had killed them and that was why I wasn't good enough.

ASTBURY: So in what way do you think your practice might have been the reason why these babies had died?

STONIER: I didn't know. I thought maybe I had missed something, maybe I hadn't acted quickly enough.

ASTBURY: Give us an example.

STONIER: I hadn't played my role in the team. I'd been on a lot of night shifts when doctors aren't around. We have to call them. There are less people and it just worried me that I hadn't called them -- quick enough or.

ASTBURY: And you felt evil?

STONIER: Other people would perceive me as being evil, yes, if I had missed something.

ASTBURY: "I'm a horrible, evil person"; that's your take on you?

STONIER: I think it's how this situation made me feel.

ASTBURY: "I don't deserve mum and dad."

STONIER: I felt so guilty that they had to go through this, that I wasn't good enough for them or any of them and it was all just becoming a big mess and I'd just be better off out of it for everybody.

ASTBURY: You put down there, Lucy, that you "killed them on purpose".

STONIER: I didn't kill them on purpose.

ASTBURY: Do you believe there's a potential that you caused their deaths?

STONIER: Not intentionally.

ASTBURY: Okay. So do you believe that you were carrying out practices that weren't competent?

STONIER: No.

ASTBURY: Okay. So where's this pressure that's led to having these feelings come from?

STONIER: I think it was just the panic of being redeployed and everything that happened.

ASTBURY: Okay.

STONIER: It makes more sense now, but at the time I did think that they might think I was incompetent, that I might have unintentionally caused something.

ASTBURY: Lucy Letby explained that she'd reviewed her competencies since being removed from the unit with Yvonne Farmer: Is that on your neonatal unit or your new unit?

STONIER: We didn't do them on the unit, we just did them in an office environment and went through all the competencies. We didn't do a practical on the unit.

ASTBURY: Okay. And that was last year?

STONIER: Yes.

ASTBURY: Okay. So I think, just to make it clear what you just said there, it was implied that your level of competency may have resulted in deaths and that's where you got all these feelings from, but the trust didn't say it directly, and you don't think that you failed with regards to your care and the competencies offered to the babies?

STONIER: That's correct, yes.

ASTBURY: Okay. Which competencies was -- could you be failing with that would result in a death of a baby?

STONIER: I suppose the thing that come to my mind was medications, because that's something that we do a lot of on the unit, and the babies are on a lot of medications.

ASTBURY: What part of your competency would you be failing with if it wasn't being done correctly? So going through the process of when you administer medicine to the baby, what part of that process would cause the death if it wasn't done correctly?

STONIER: The wrong drug or the wrong dose.

ASTBURY: Are there any other competencies that you might think, if you didn't do that correctly, it could cause a problem with the baby?

STONIER: Maybe if I wasn't competent with a piece of equipment.

ASTBURY: And do you feel competent with all the equipment you use?

STONIER: Yes.

ASTBURY: With regards to your parents, you mentioned, "I don't deserve mum and dad". Is that purely in relation to the problem you were having on the unit and being removed?

STONIER: Yes.

ASTBURY: Okay. So was nursing something that they were particularly proud that you were doing? So tell me about that.

STONIER: Well, it was -- it was a big thing. I was the first person in the family to get into university and to move away and come and do nursing and, yeah, they were really pleased. So I just felt anything like this -- well, anything that's in the note, they'd be disappointed, and they were. They were really, really upset about it.

ASTBURY: What were they disappointed and upset about?

STONIER: That I'd been removed from the unit.

ASTBURY: Did you need to tell them?

STONIER: Yeah.

ASTBURY: Why?

STONIER: I didn't want to lie to them.

ASTBURY: Okay. The only other thing is that in terms of, I think, within that note, you were questioning maybe, "What does the future hold?" What were your thoughts around that?

STONIER: I think I just didn't know what was going to happen. It just all overwhelmed me at the time. It was hard to see how anything was ever going to be okay again.

ASTBURY: So moving forward prior to this point, what did you envisage your life being, moving forward?

STONIER: I was very career-focused.

ASTBURY: Right.

STONIER: And I was worried that all of this would stop anything like that, that I'd lose my job or that it'd just be on my record, other people would change their opinion of me.

ASTBURY: Lucy Letby described her family with whom she was very close and she was asked: Okay. You then go on to say in your notes Lucy, "The world is better off without me". What do you mean by that?

STONIER: That they'd all be better off without me.

ASTBURY: Why?

STONIER: Because I disappointed them.

ASTBURY: And in capital letters, "I AM EVIL, I DID THIS".

STONIER: Because that's how it had all made any feel at the time.

ASTBURY: That you'd done something wrong?

STONIER: Yeah. Not intentionally, but I felt if I'd done something, if my practice wasn't good enough or people didn't think I'd done something in the right way, then it made me an evil person because I couldn't do the job properly.

ASTBURY: "I am an awful person and I pay every day for that right now."

STONIER: Because I felt like I was having to pay for something that I didn't do, being away from my jobs and my friends and having to go to a new area where I didn't know anyone.

ASTBURY: So this is all how others are making you feel and how you were feeling yourself?

STONIER: Yes.

ASTBURY: "I'll never have children or marry. I'll never know what it's like to have a family."

What did you mean by that, Lucy?

STONIER: Just that I'd never meet anybody and therefore I'd never have a family.

ASTBURY: Why did you think that?

STONIER: Because nobody would want to -- if you say to somebody you had to be redeployed, then people make assumptions, don't they, and if my practice had caused these problems then I wouldn't deserve to have children myself.

ASTBURY: Purely because you had been redeployed off one unit?

STONIER: Yes, because at the time it was huge.

ASTBURY: You then put down:

"I hate myself so much for what this has -- I did this, why me?"

STONIER: Again, I was made to feel I had done it through not being not competent.

ASTBURY: Did what?

STONIER: Well, did something that -- that had led to these babies collapsing, dying.

ASTBURY: Did you ever consider that it might have nothing to do with you or your incompetency?

STONIER: Not that moment in time, I just...

ASTBURY: Okay. What about now?

STONIER: No, I don't feel it is my competencies.

ASTBURY: So what changed between these kind of thoughts and now that you're confident that your competencies weren't lacking enough to cause any serious collapse or death?

STONIER: Time. And I've re-done my competencies and had that grievance procedure and nothing was sort of raised through that or any of the other investigations that have taken place to sort of suggest that I'd been incompetent in something.

Sorry: That I hadn't been competent in something.

ASTBURY: "No hope, despair, panic, fear, lost."

Is that how you felt, you had no hope?

STONIER: It just made me feel like no hope for anything, yeah.

ASTBURY: If you knew that you'd done nothing wrong?

STONIER: Well, at that point I was made to feel that maybe I had, so I was worried that maybe I had in terms of my practice and my competencies.

ASTBURY: Who had made you think that?

STONIER: The trust.

ASTBURY: Lucy Letby explained that she was informed on behalf of the trust that she was being redeployed as her competencies were an issue: Has anyone ever said that you have done something wrong?

STONIER: I found out via the grievance procedure and the Royal College of Nursing that some of the consultants had made comments.

ASTBURY: The comment that's in there referring to the note presumably, "I did this. Why me?", what does that refer to?

STONIER: That I just caused the disappointment.

ASTBURY: What's the "why me"?

STONIER: I felt -- well, why was it happening to me? Because at the time they were saying that I was a common feature but so were other staff.

ASTBURY: Okay.

STONIER: But then it was only myself that was redeployed, so obviously but why me, why is it just me that it's happening to?

ASTBURY: What was the "I did this"?

STONIER: The upset and everything that I caused those people.

I felt that it was me, not intentionally but through that situation, through the redeployment.

ASTBURY: Lucy Letby then explained that the period up to her redeployment had been a low point: Okay, what made the first part of 2016 so challenging then?

STONIER: Well, just reflecting all the year that we had had before and I think it just affected morale on the unit.

We were all feeling -- it's a shock, we're not used to deaths like that. And when you're involved with them...

ASTBURY: Okay. At which point did the unit start to feel like that?

STONIER: I'd say about earlier in the year, perhaps January.

ASTBURY: January. Why particularly then? What had happened?

STONIER: I'm not sure specifically, it is just with it being the New Year and things people just were hoping for a better year and then things happened again.

ASTBURY: "Things happened again", what do you mean?

STONIER: We continued to have sick babies and lost some babies.

ASTBURY: Were there any in particular that you lost that you recall?

STONIER: When? At that time period?

ASTBURY: Yes.

STONIER: I can't remember specifically then, no.

ASTBURY: When you say "we" -- you refer to "we were feeling quite low". Who were you referring to as the "we"?

STONIER: The nursing team.

ASTBURY: Who do you class the nursing team as, everybody?

STONIER: Yes, the nurses and the nursery nurses on the unit.

ASTBURY: You talk about the babies being specifically sick.

What was the difference from another year?

STONIER: I think we were seeing more babies who had complex needs, we were having babies with chest drains that we don't get very often, babies with stomas that we don't care for a great deal. We had quite a few that were quite extreme prem babies with congenital abnormalities, a lot of twins and then we had the triplets.

ASTBURY: In terms of emotional outlets for coping, you know, your coping mechanisms, what would you use?

STONIER: Usually just talking things through with the team or with my friends.

ASTBURY: Is this an emotional outlet, doing things like this?

STONIER: Yeah.

ASTBURY: Right. Okay. Do you use social media and stuff?

STONIER: Yes.

ASTBURY: And that's the way that you used to speak to your friends?

STONIER: Some of them, yes.

ASTBURY: Okay. We'll take a break there and I think the time is 24 minutes to 9.

The interview concluded.

STONIER: Yes.

ASTBURY: If we go to the next overarching interview, please, that took place on 5 July 2018, 2 days later?

STONIER: Yes, that's correct.

ASTBURY: It began with introductions and: Okay. In terms of the investigation, and obviously this is your opportunity, is there anything that you feel us, us as an investigation, need to look at concerning the amount of deaths and collapses over a short period of time?

STONIER: I think the staffing maybe needs -- I'm not saying that staffing has caused it, but I think staffing levels were quite poor at times with an inadequate skill mix sometimes.

ASTBURY: Okay.

STONIER: And I think a lot of people, like myself, were doing a lot of additional shifts and overtime and having shifts changed round at short notice. I think a lot of people were feeling the strain physically and emotionally. I don't think a lot of support was offered to the team throughout this event with the deaths and things. There's also some issues with the unit just in terms of it's very small, we don't always have the equipment that we need, we have to go and get it from other units, or are pushed for space and trying to look after sick babies in not always ideal environments and I personally just found during this that there wasn't always a very clear and supportive sort of management, structural, medical support particularly towards nursing staff. That's a personal opinion.

ASTBURY: How was staffing levels different during that period than they were a month before [Baby A] collapsed and died and a month after [Baby Q] collapsed and died [it says], for example?

STONIER: I don't recall specifically, but often sort of from May, June onwards, we are short of staff due to people taking more holidays.

ASTBURY: Right.

STONIER: And I remember at that time we had a lot of new starters that had just started on the unit, so we were quite bottom-heavy in terms of having more inexperienced staff that needed support on the unit -- and I think we also had a couple of members of staff that were on long-term sick leave during these times as well.

ASTBURY: Okay. So do you think any of these deaths and collapses occurred due to poor care?

STONIER: I don't think anybody intentionally gave poor care, but I think maybe if staffing had been better people may not have been caring for as many babies at once or would have had different shift patterns, maybe, or the doctors would have been more readily available.

ASTBURY: What about equipment? Do you think any of these babies had collapsed or died because of the equipment that was around or the lack of equipment?

STONIER: I think there's been delays with them having some of the support that they need because we've had to go and get equipment, yes.

ASTBURY: Would any of the lack of equipment or staff cause the collapse of a baby, the initial collapse?

STONIER: No, I don't think it would cause the collapse, no.

ASTBURY: It's clear that the babies that we've been speaking about over the last few days we're saying aren't just unexpected but suspicious.

STONIER: Right.

ASTBURY: Do you understand that?

STONIER: Yes.

ASTBURY: That's the initial collapse --

STONIER: Yes.

ASTBURY: -- as opposed to subsequent collapses.

STONIER: Okay.

ASTBURY: If you say lack of staff, lack of equipment, doctors not reacting maybe as quickly as they should do, can you apply any of those three factors to the babies that we've spoken about here?

STONIER: Yes, for some of them I think if staffing had been better then maybe there would have been more people around for that baby.

ASTBURY: And who -- can you recall who they were specifically?

STONIER: I think [Baby Q] is one because I was stretched between two --

ASTBURY: Yes.

STONIER: -- nurseries which is not ideal.

ASTBURY: I think you alluded to that in the interview for him, yeah.

STONIER: I recall the day that I had [Baby G] and she was down in nursery 4 and I had a number of other babies at that time as well.

The day with [Baby M], the nursery was very busy in nursery 1 and he was not in a correct space. Either he was in just parked in the corner which -- it wasn't ideal.

I don't remember -- and then I just remember we had a lot of junior staff that we were supporting during that time as well.

ASTBURY: Okay.

And Ms Letby's solicitor said: I think that when you gave the initial interview with regard to [Baby P], I think you described that as quite chaotic when they were actually trying to --

STONIER: Yes.

ASTBURY: -- resuscitate.

Then the officer says: I understand that. They are certainly factors that could affect every walk of life, aren't they, but what we are saying is that we are treating the babies' collapses and deaths as suspicious; you understand that, don't you?

STONIER: Yes.

ASTBURY: Okay. In general terms, the investigation's looking into a number of deaths between 2015 and 2016 and other babies who have collapsed and survived. So a direct question is: between those dates and that amount of babies, have you done anything to intentionally harm those babies?

STONIER: No.

ASTBURY: When did you first become aware that there was an unnaturally high rate of mortality on the unit?

STONIER: In a formal way it was said to me by the unit manager - - I think in the May 2016.

ASTBURY: Okay. What do you mean "in a formal way"?

STONIER: Well, she took me into the office and I think it was at that point I was moved on to day shifts and she explained that there had been an increased rate and she was currently working on some tables to work out the statistics.

ASTBURY: Okay. So informally when did you have the realisation or were told that this is really an unnaturally high level of mortality for Chester's unit?

STONIER: I think at the very beginning when we lost the three babies, when we lost [Baby A], to have three so quickly, that in itself was unusual and it was probably more deaths than we usually have.

ASTBURY: In a year?

STONIER: Mm.

ASTBURY: Okay. In that first month, I think from what you were saying earlier, that's more deaths than you've experienced since you've worked in neonatal?

STONIER: I think so, yeah.

ASTBURY: Okay. When you were first made aware of the investigation that the hospital were doing, were you told specifically the names of the babies that they were investigating?

STONIER: No.

ASTBURY: So even the ones here, the ones that resulted in death for example, were you told formally by them?

STONIER: No, no.

ASTBURY: Okay. In terms of the investigation from the Countess' point of view, but also from the police investigation, have you done any form of research into any of the babies or any of the deaths?

STONIER: In what way do you mean research?

ASTBURY: For example, you know who died because you were there or who collapsed, you're aware of the babies' names. When you were still on the neonatal unit would you research their medical notes, for example, that sort of thing, kind of thing?

STONIER: I think I'd reviewed their medical notes, yes, at some point, yes.

ASTBURY: And what was the purpose of that?

STONIER: Just as a recap, really, to think -- to take things in better when it's not happening at the time.

ASTBURY: Okay. For what purpose?

STONIER: I think it just helps to go back in to read what happened, so obviously you have it clear in your mind that everything was done.

ASTBURY: At the time of collapse or death, you mean, or as a result of the subsequent investigation?

STONIER: What do you mean, sorry?

ASTBURY: All right then, take [Baby A]. Did you do any research yourself with regards to [Baby A]?

STONIER: So did I access his notes after he died?

ASTBURY: Yes?

STONIER: I might have done. I don't recall specifically.

ASTBURY: Okay. All right then, any of these babies that you looked into after death or collapse, what was the purpose of that?

STONIER: Just for clarity and for sort of my own debrief as such, just to recap.

ASTBURY: How close to the death or collapse was that?

STONIER: I don't remember.

ASTBURY: Okay. Was that research as a result of the investigation launched by the hospital?

STONIER: No, I'll not sure. I might have looked after and before. I might have done that prior to the investigation, I'm not sure.

ASTBURY: Okay. With regards to the police investigation, at which stage did you become aware of the babies' names that we were investigating?

STONIER: I don't think I did until now.

ASTBURY: Okay. So on 7 April you were moved to a day shift and you've kind of told us how that made you feel. You said that you felt that people's attitudes towards you had changed and you'd doubted your own capabilities. Is that a fair --

STONIER: Yes.

ASTBURY: Okay. So you were moved on to days. And after you were moved on to days in the June, as we've just discussed, [Baby O] and [Baby P] both died and [Baby Q] collapsed.

So what are your thoughts on that?

STONIER: That they have collapsed?

ASTBURY: Yes. After you've been swapped on to days.

STONIER: I am not sure.

ASTBURY: Okay. So a lot of the collapses and deaths prior to you getting moved on to days have been during the night-time, on a night shift.

STONIER: Yes.

ASTBURY: Okay. After you get moved on to days there are two deaths and a collapse within 3 days of each other.

STONIER: Yes.

ASTBURY: Okay. Do you have any comment to make about that?

STONIER: I can't explain that, no.

ASTBURY: Do you have anything in your possession which relates to any of the allegations for which you've been arrested?

STONIER: What do you mean? Sorry.

ASTBURY: Paperwork, medical records, anything.

STONIER: No. Not that I know of, no.

ASTBURY: Okay. Have you ever taken anything relating to the babies that we've discussed home?

STONIER: No. I don't know if -- I might have sometimes taken handover sheets accidentally home with me.

ASTBURY: Okay.

STONIER: Not medical notes, no.

ASTBURY: No. Not just sticking to medical notes, anything relating to --

STONIER: I don't know specifically to them. I think sometimes I have brought handover sheets home, yes.

ASTBURY: Why? What's the purpose of that?

STONIER: Just inadvertently. They've just been left in my pocket.

ASTBURY: Okay. And I think we asked you sort of a little bit throughout whether you would take any mementos from the babies yourself and I think you said no; is that right?

STONIER: No.

ASTBURY: I just wanted to ask you a few more things about the note, NAC10. Did you write all that at the same time?

STONIER: I don't remember specifically, but I think so.

ASTBURY: Okay. Is there a reason why it's written in that format? You see that some of the writing is to one side and some on the edge of the page.

STONIER: I think I've just done it when I was very upset and it all just kind of come out at once in different ways.

ASTBURY: Okay. And where were you when you wrote that?

STONIER: At home.

ASTBURY: What was going through your mind at the time?

STONIER: I just felt like I'd let everybody down, that I'd let myself down, that people were changing their opinion of me, that I thought I'd lost my job and I was isolated from my friends.

ASTBURY: And just confirm when you think roughly, the time, month year?

STONIER: I know it was after when I'd been -- I'm not sure of the exact time but it was some time after I'd been removed in July 2016.

ASTBURY: You particularly got the word "hate" there. I'm right in saying that's the word "hate"?

STONIER: Yes.

ASTBURY: Which is circled with a big black circle, "hate" in bold letters. What's the significance of that?

STONIER: That I hate myself for having left everybody down and for not being good enough.

ASTBURY: And just confirm to me why you think that you're not good enough when you wrote that down?

STONIER: Because I'd just been removed from the job I loved, I was told that there might be issues with my practice, I wasn't allowed to speak to people, I was having to do a job that I didn't enjoy with people that I didn't know.

ASTBURY: And this was within a couple of months of being removed?

STONIER: Yes, I think so, yes.

ASTBURY: And all these emotions, these feelings that you put on this stage, had this come to a head?

STONIER: Yes.

ASTBURY: Had anything triggered on this particular day for you to write that?

STONIER: I don't recall specifically, no.

ASTBURY: Have you ever shown that note to anyone?

STONIER: No.

ASTBURY: Can we have a look at that for me again and where you specifically say, "I don't deserve to live, I killed them on purpose". Can you explain to me again what you actually meant by that?

STONIER: That -- that's how I was being made to feel, that if my practice hadn't been good enough and I was linked with these deaths, then it was my fault and I had done it and they thought that I was doing it on purpose, not that I had done it on purpose, but that's how I was made to feel.

ASTBURY: Specific words:

"I killed them on purpose and I'm evil. I did this. [And] I'm an awful person. I pay every day for that."

STONIER: It's because I felt I was awful because I -- I maybe hadn't been good enough.

ASTBURY: You're being very hard on yourself there if you haven't done anything wrong.

STONIER: Well, I am very hard on myself.

ASTBURY: "I did this. Why me? I did this."

What did you do?

STONIER: I felt that I wasn't good enough. That's -- that's what they were implying, that I hadn't -- that my competencies hadn't been good enough, they were removing me. I felt that I had -- bad person, I wasn't good enough, I had caused them, I had caused them to think that.

ASTBURY: "That I did this." What is this?

STONIER: I don't know. I felt the situation had been caused by them implying that, that I hadn't been competent.

ASTBURY: Lucy, were you responsible for the deaths of these babies?

STONIER: No.

ASTBURY: Okay. We shall take a break.

Then the time is given and the interview concludes.

STONIER: Yes.

ASTBURY: Two of those interviews we've heard are on the first occasion when Ms Letby was arrested. The next interview is 10 June 2019.

STONIER: Yes.

ASTBURY: So this interview would have taken place during the second arrest; is that correct?

STONIER: Yes, that's correct.

ASTBURY: It begins with the introductions, caution and the explaining of legal rights.

STONIER: Yes.

ASTBURY: Lucy, prior to starting this interview you've mentioned before about a handover process that takes place at the start of your shift with the nurse previously; is that correct?

STONIER: Yes.

ASTBURY: Okay. Are you given any documentation during that handover?

STONIER: Yeah, we have a handover sheet of -- of the patients that are on the unit at that time.

ASTBURY: Okay. Explain the purpose of those handover sheets.

STONIER: Well, to relay information between staff so that each member of staff's got the brief outline on each of the babies.

ASTBURY: Okay.

STONIER: Then we get a more in-depth handover on your own baby.

ASTBURY: Who has a copy of this handover sheet?

STONIER: All members of staff on the unit.

ASTBURY: Where are they kept during the shift?

STONIER: In our pockets -- in the staff's pockets.

ASTBURY: Why's that?

STONIER: So we can make reference to it throughout the shift if we need to.

ASTBURY: Okay. And when you were personally given handover sheets, Lucy, what did you used to do with yours?

STONIER: Keep it in my pocket for the shift.

ASTBURY: And when you finished your shift, what would you do with the handover sheets?

STONIER: Um, ideally put it in the confidential waste bin.

ASTBURY: And why would that be?

STONIER: For confidentiality, so the public can't pick up the sheets.

ASTBURY: Mm-hm. Then where's that situated Lucy?

STONIER: On -- by the nurses' station.

ASTBURY: Okay. Is that what you would do with your handover sheets?

STONIER: Yes. Not every time though. There have been times when they've come home with me.

ASTBURY: Okay. Is there a policy in place around handover sheets, Lucy?

STONIER: Not that I know of.

ASTBURY: What does generally happen to them then with the other colleagues on the unit? What do they do with them?

STONIER: They put them in the confidential waste.

ASTBURY: Is that at the end of the shift?

STONIER: Yeah.

ASTBURY: Okay. So there's no filing system for them at all?

STONIER: No, they're just discarded at the end of the day by that member of staff.

ASTBURY: Okay. When you were previously arrested, Lucy, you were aware that your home address was searched and a large quantity of these handover sheets were found at your home address. Can you explain that?

STONIER: They're just sheets that have inadvertently come home with me in my pocket. I have no emptied my pockets before coming home.

ASTBURY: Okay. Can you explain why you kept these at your home address?

STONIER: Um, no. There's no specific reason. They just came home with me and I didn't do anything with them.

ASTBURY: Can I ask what you actually wear when you're a unit?

STONIER: A set of scrubs, so a pair of trousers and then a tunic top that's got two pockets here and a pocket at the top.

ASTBURY: So which pocket would you put the handover sheet in?

STONIER: One of the bottom pockets.

ASTBURY: Bottom. Either left or right or?

STONIER: I don't remember having a specific pocket --

ASTBURY: Okay.

STONIER: -- that I put it in.

ASTBURY: And tell me at what point when you got home did you realise that you were still in possession of these handover sheets?

STONIER: When I have got home and taken my uniform off.

ASTBURY: So talk me through then when you have taken your uniform off and you've found these handover sheets, what did you do with them?

STONIER: I just put them all in one area.

ASTBURY: Which area was that?

STONIER: They were all together in a folder in the spare room.

ASTBURY: Okay. Explain to me why you put them all together in a folder?

STONIER: Because I didn't know how to dispose of them, so I didn't dispose of them.

ASTBURY: You didn't know how to dispose of them?

STONIER: No.

ASTBURY: Whose permission did you have, Lucy, to remove these handover sheets from the hospital?

STONIER: No one's.

ASTBURY: Who else knows you've got them at your home address?

STONIER: No one.

ASTBURY: Have you shown them to anyone?

STONIER: No.

ASTBURY: Whilst they've been in this folder at home, what have you used them for?

STONIER: I haven't.

ASTBURY: How often have you looked at these handover sheets, Lucy?

STONIER: Hardly ever.

ASTBURY: Did those sheets that are in your folder that you've kept at your home address, Lucy, relate to babies which you were the designated nurse for?

STONIER: Yes, they're all babies that are on the unit at that point, whether you look after them or not, so yeah.

ASTBURY: Okay. Have you ever previously taken any of these handover sheets home and disposed of them?

STONIER: No, I don't think so, because I haven't got a shredder and that's how I would -- that's how I would have to get rid of them.

ASTBURY: Okay. So why would you have only kept some of the handover sheets in a folder, Lucy?

STONIER: Because they're just the inadvertently ones that have come with me [as read].

ASTBURY: Have you retained in any way any other documentation from the hospital of any description?

STONIER: No. I have some printed-out policies --

ASTBURY: Okay.

STONIER: -- but I don't know if that's not allowed.

ASTBURY: Have you retained any other confidential documentation at home?

STONIER: No.

ASTBURY: Have you retained any other documents from any other hospitals that you've previously worked at?

STONIER: Again, I've -- I've got policy sheets from different hospitals, but not patient information.

ASTBURY: When you say policy sheets, describe them to me.

STONIER: Like guidelines for how different hospitals do things, I've printed them off and brought them home for assignments and things.

ASTBURY: So specifically what policy sheets are you referring to?

STONIER: I think I've got some on -- loads because I did my ITU course and we had to have policies for a lot of the -- so I've got things on feeding, on jaundice, on hypoglycaemia, on necrotising enterocolitis, I've got various.

ASTBURY: Okay. Where are those policies kept that you've printed off?

STONIER: Um, some are within the -- my intensive care folder, some are just loose. I'm not sure exactly where all of them are.

ASTBURY: Okay.

Then you ask your colleague: Do want to ask anything?

You say that the handover sheets that you put in your pocket relate to being -- to you being a designated nurse for these babies; yes?

STONIER: So the handover sheet has every baby on the unit at that time.

ASTBURY: Right, okay.

STONIER: And it's not just the baby you're looking after, it's every baby.

ASTBURY: Would you have had cause to take some out of the waste, Lucy?

STONIER: Out of the clinical waste? No.

ASTBURY: Okay. So just to confirm, Lucy, when I've asked you why you decided to keep the handover sheets, you've confirmed that you weren't aware, didn't know how to dispose of them, therefore you kept them in a folder?

STONIER: Yes, at the time I've got home, realised they're there, and I've just not done anything about it.

ASTBURY: Moving on, Lucy, I would likes to talk to you about your mobile phone and telecoms. Would you have used it at work?

STONIER: Yes.

ASTBURY: Okay. Is that permitted? Is there any issue about allowing you to use it at work?

STONIER: We're advised not to use it, like, near to the patients, but on breaks and out of the clinical area.

ASTBURY: Where would you keep whilst you were at work?

STONIER: Either in my pocket or in my bag.

ASTBURY: Lucy Letby could not recall the exact device she would have had in 2015 and 2016, but it would have been an Android with access to social media and she was asked: Okay. Does anyone else have access to your phone?

Do you give it out to anyone or lend it to anyone?

STONIER: Not particularly no.

ASTBURY: Okay. So you obviously use your phone at work during work time. If you've got any -- a bad day or issues going on at work, who would you sort of use your phone to contact? Who'd be your first port of call?

STONIER: Um, a friend.

ASTBURY: Any particular close friend that you would use your phone to?

STONIER: [Nurse E].

ASTBURY: Okay. And [Nurse E], she works at the [redacted]?

STONIER: Yes.

ASTBURY: Okay. Are there any other close friends that you would contact or your family?

STONIER: I've got a couple of different close friends over the years that I probably would have contacted, yeah.

ASTBURY: Right, okay. And how often would you contact them in regards to anything that was going on at work? Would that be frequently?

STONIER: I'm not sure. It would depend on what was going on at the time.

ASTBURY: Lucy Letby confirmed that she would use WhatsApp, text or Facebook Messenger, not iMessage, as she didn't have an iPhone: Did you discuss the welfare of babies at all with any of your friends?

STONIER: Um, oh yeah, I've discussed patients at times, yeah.

ASTBURY: Okay what sort of things have you discussed?

STONIER: I'm not sure exact details now. I've communicated with friends when babies have been unwell or if they've passed away.

ASTBURY: Right. So would that be sort of straightaway or within the same sort of shift a few hours later?

STONIER: I'm not sure, I can't --

ASTBURY: So, you know, we discussed, the first time you were brought here and arrested, and the babies you were involved in the care of.

And then Lucy Letby was nodding her head. It's not recorded, obviously, audibly: So would you have contacted friends following those?

STONIER: Yes.

ASTBURY: And how often would that communication go on for generally?

STONIER: About the babies specifically?

ASTBURY: Yeah.

STONIER: I'm not sure.

ASTBURY: Would there be a purpose for you doing that, contacting friends?

STONIER: Yeah, they were -- they're my support network.

ASTBURY: So did that make you feel better when you communicated with them?

STONIER: Yeah and it was somebody in the same profession that could -- rather than speaking to a family member who didn't understand the unit and things, it's helpful to speak to a colleague.

ASTBURY: Did you discuss theories about what was going on?

STONIER: I'm not sure, possibly.

ASTBURY: Or individual patients?

STONIER: I don't know. Possibly.

ASTBURY: What about family members? Did you communicate with them at all?

STONIER: Yes, I used to speak to my parents every day after I'd finished work -- well, every day, anyway, but...

ASTBURY: Okay. And after the collapse of a baby, which family member would you turn to?

STONIER: My mum.

ASTBURY: For the same reasons, to help you get through?

STONIER: Well, for her support -- I wouldn't talk to her about it in the level of detail that I would with a colleague.

ASTBURY: So can you just describe to me how it made you feel, discussing this with friends and family, how it sort of helped you with the whole process?

STONIER: I suppose I just saw it as -- that was a safe way of me sort of offloading how I felt to somebody I trusted.

I wasn't somebody that would go home -- I lived alone.

I wasn't somebody that would go and necessarily seek out somebody to speak to in person. That was my way of thinking through things.

ASTBURY: Okay. And did it help?

STONIER: Yes.

ASTBURY: In what way?

STONIER: Well, because they would have been supportive or, you know, share -- a nurse knows how you feel when things happen and it's just having that common ground with somebody and a bit of support from them.

ASTBURY: Okay. Did you ever seek advice regarding the treatment of a baby or what was going on through the use of your phone through social media?

STONIER: No, I don't think.

ASTBURY: As in one of your colleagues who might be experienced?

STONIER: Um, I'm not sure. I think I rang -- um, had ran some things past one of the doctors that I was friendly with at the time.

ASTBURY: Who was that?

STONIER: [Dr A].

ASTBURY: Okay. And what sort of advice did he give to you?

STONIER: Just, I suppose, reassurance. Just somebody on another level to talk to about what was happening or if I was having a difficult day.

ASTBURY: So you'd -- she'd be the first person you'd turn to and after [Nurse E]?

STONIER: Well, at different times [Dr A] was -- I was close to [Dr A] in the later stages. I had other friends: [Nurse A], Minna Lappalainen.

ASTBURY: Okay. So you've committed with all those over the years?

STONIER: At some point, yes.

ASTBURY: And this would be during and after work?

STONIER: Yes.

ASTBURY: Is there a reason why you wouldn't get advice or support face to face?

STONIER: We get support sometimes on shift, but it would depend who you were working with and what was going on in the unit, and who it was that -- well, whether you felt able to talk to that person or not. When we've had a difficult day on the unit, a baby's been unwell or it's been particularly busy, I don't know, somebody had phoned in sick or anything that was a bit different on the unit, out of the normal, I might seek support from somebody.

ASTBURY: Okay. And when you were asked about occasions that you have messaged colleagues for advice relating to work, you have said it was for reassurance. Explain what you meant by that.

STONIER: I can't remember specific -- but I know I -- I've mentioned [Dr A] before now in terms of when we'd lost certain babies. I know he'd gone to like debriefs and different things that nursing staff weren't invited to and I think I checked some different policies with him over time.

ASTBURY: And explain why you were particularly interested in those debriefs.

STONIER: Because they were babies that I'd had involvement with --

ASTBURY: Okay.

STONIER: -- or been present for.

ASTBURY: Okay. And you said that you weren't invited to these debriefs; is that correct?

STONIER: Not all, some. Some of them.

ASTBURY: Right?

STONIER: Some you're not and then there's things were discussed at medical level only and things, so...

ASTBURY: Okay. The next area I want to talk to you about Lucy is your training and, correct me if I'm wrong, but our understanding from the investigation is you qualified as a band 5 nurse some time in 2012. Can you confirm if that's correct?

STONIER: September 2011.

ASTBURY: Okay.

STONIER: And I started working on the unit January 2012 and that was my first job.

ASTBURY: Lucy Letby discussed her training in administering blood transfusions and blood components, her mentorship for students, and acquiring credits towards a master's qualification. She explained that she had qualified in specialty training at Liverpool Women's Hospital in February 2015.

She was asked: Okay, during the training, obviously, you have described to me what it involved and the competencies.

What about any risks or dangers dealing with neonatal babies? Were you taught anything specifically in relation to that?

STONIER: Yeah, we had different lectures and things about different neonatal conditions.

ASTBURY: Mm-hm.

STONIER: We spent time going out with the resus coordinator -- we had somebody that is on shift that attends any collapses or resuscitations or births at that point and we spent time with that person to go out and get experience of the acute sort of emergency setting.

ASTBURY: And how did you find that?

STONIER: Just very different to Chester. It's just not something that we would see and do and they're sort of like -- I went to a lady that was delivering in the corridor and things. That's just something that I'd never seen before.

ASTBURY: So all these areas was (sic) knowledge you could potentially bring back --

STONIER: Yes.

ASTBURY: -- to the unit?

STONIER: Yes.

ASTBURY: And amongst the staff on the neonatal unit, Lucy, were there any other nurses of band 5 who'd done this training?

STONIER: Yes, there was myself and Bernie Butterworth. We were the only two --

ASTBURY: Okay.

STONIER: -- which is why I found I was quite often allocated these babies because I was on shift with people that didn't have the ITU course and therefore weren't able to care for them.

ASTBURY: Yes.

Lucy Letby described further training in basic life support and infection control, breastfeeding support and annual neonatal updates: Okay, moving on, Lucy. In May 2015 there was a competency assessment for "Safe administration for medication by bolus/intermittent via a long line, Broviac line or umbilical venous catheters" [as read].

Do you recall that training?

STONIER: Yes.

ASTBURY: Can you explain to me what that involves?

STONIER: Okay, so we didn't have any training as such, it came from -- when you've done the intensive care course, you are then

eligible to access these sort of lines and to do the competency. So usually you would just work with another nurse and then they would support you and watch you in drawing it up and preparing whatever needs to be given via that line. Then there's a competency -- of questions that they ask you as well.

ASTBURY: Okay. So did you say, sorry, that there wasn't a specific training?

STONIER: So there wasn't any -- no, there wasn't a specific --

ASTBURY: Right.

STONIER: -- training aspect. No, it's just something you sort of learn on the job.

ASTBURY: And how long does that take place for?

STONIER: I think you'd have to be watched three times, if I remember correctly.

ASTBURY: Okay. And do you recall who you were assessed by?

STONIER: I think one was Chris Booth.

ASTBURY: Mm-hm.

STONIER: Somebody, [Nurse A], I can't remember.

ASTBURY: And explain to me how, this training, you would then apply it to your role?

STONIER: I'd then be able to give baby medications via these sorts of lines. Rather than just being a second checker I would actually be able to --

ASTBURY: Okay.

STONIER: -- have access to those lines.

ASTBURY: Okay. And how often would you then use that method, so be able to give medication?

STONIER: Quite frequently.

ASTBURY: Mm-hm?

STONIER: Most of the babies on the unit have some form of central access and when you're new to having learnt something, they are usually quite keen for you to get as much experience as you can --

ASTBURY: Yes.

STONIER: -- so you end up doing a lot of the drugs and things.

ASTBURY: Okay. How did you find that?

STONIER: Okay, I think it was certainly very different. It was very different learning about those separate lines to just a normal peripheral line. Obviously there's a little more risk and sort of learning. You have to learn where the line placement is in terms of X-rays a little bit and it's more responsibility.

ASTBURY: When asked about the risks involved, Lucy Letby identified infection, the line moving or the line leaking and was asked: Okay. And having done the training, would you class yourself as competent in that area?

STONIER: Yes.

ASTBURY: Is there any part of that training, Lucy, that you're not that happy with?

STONIER: Um...

ASTBURY: Or are you fully confident with?

STONIER: I think the only thing we -- we don't see a lot of babies on the unit with a Broviac line.

ASTBURY: Okay. Moving on, Lucy, you've also completed in May 2015 assessments for the safe administration of medication by bolus and by bolus [as read] and also safeguarding children as well. I'm guessing those are two separate areas of training?

STONIER: Yes.

ASTBURY: So the first area then, the safe administration of medication, what can you tell me about that?

STONIER: I don't remember that training specifically.

ASTBURY: Did you do or did you attend any specific resuscitation training for neonatal babies?

STONIER: Yes, we attend the neonatal life support programme.

That's done every 4 years. That's lasts for 4 years.

ASTBURY: And what did that training involve?

STONIER: Resuscitation scenarios and skill stations and at the end of the day you're assessed. Then you get called through and it's sort of like a random scenario and you have to manage that.

ASTBURY: Is there any other training, Lucy, that you received while you're a nurse on the neonatal unit that I haven't gone through with you?

STONIER: I attended an IV study day at Alder Hey.

ASTBURY: When was that?

STONIER: That's when I first qualified to be able to give medications via a line. That had a competency assessment. And I've attended various study days, but they were just for my own --

ASTBURY: Yeah.

STONIER: They weren't assessed study days.

ASTBURY: Okay.

STONIER: I don't think there's anything else that I've been assessed in.

ASTBURY: Is there any training that you've failed at all, Lucy?

STONIER: No, not that I'm aware of, no.

ASTBURY: Okay. In relation -- we've touched on it before when speaking to you, Lucy, in relation to insulin training.

Tell me about any specific training you've had about that.

STONIER: Well, I don't recall having any specific training in insulin specifically, no.

ASTBURY: Have you received any inputs around it?

STONIER: Um, hypoglycaemia and hyperglycaemia. It isn't something that's really discussed at updates, no.

ASTBURY: So explain to me then how you become aware of how to deal with a situation involving hypoglycaemia then?

STONIER: Through just experience, experiencing it on the unit, and from when the different pathways that come out.

Usually they did change the pathway a couple of times, then you get a little bit of an email sent round, maybe with a new

policy, but then you would have to wait until you had a baby to then sort of fully get your head round it.

ASTBURY: Okay. And you've mentioned to me these pathways.

Describe to me how you're taught about them.

STONIER: You're not really taught about them, they're just sort of uploaded to the guideline system.

ASTBURY: Right.

STONIER: You're told if there's any changes and you're expected to go and look and --

ASTBURY: Okay.

STONIER: -- and familiarise yourself with anything.

ASTBURY: And what about air embolisms, Lucy, did you receive any training in relation to those?

STONIER: No.

ASTBURY: Okay were you aware of them or?

STONIER: Not really, no.

ASTBURY: Have you heard of them before?

STONIER: Um, yeah.

ASTBURY: When was that?

STONIER: I've heard of them more from an adult perspective.

ASTBURY: And tell me what that was in relation to.

STONIER: I don't know specifics. Like sometimes we've had mums on the unit who have been unwell and it's been found they've had a PE, pulmonary embolism, so that's just how I've heard of it, via that.

ASTBURY: Specifically whilst working on the neonatal unit have you ever come across it before?

STONIER: No.

ASTBURY: Then you ask your colleague: Is there anything you want to ask?

And they say: Has the air embolism training ever popped up in respect of dangers with other training that you might have had, done things incorrectly?

STONIER: Not that I can think of specifically.

ASTBURY: No? Or any of your sort of general nursing training before you qualified?

STONIER: It's been mentioned in terms of line care: you'd have to be mindful that you don't leave a line open and things like that.

ASTBURY: Mm-hm.

STONIER: But it's not something that's discussed frequently in any detail.

ASTBURY: When you say line care, you needed that competency assessment in May 2015 that we talked about, the safe administration of medication by the different lines.

Is that the type of training that you're referring to?

STONIER: Yes. I'm not sure if that's on the list or not.

ASTBURY: Okay. And have you had any concerns during care duties, what's the protocol if you had concerns in relation to your baby?

STONIER: You'd escalate it to a band 6 or the shift leader.

ASTBURY: Okay.

STONIER: And they would take it from there usually.

ASTBURY: Mm-hm. Did you feel comfortable in doing that in your role?

STONIER: Yes, sometimes. It would depend who the member of staff was. Some people are more amenable than others but I think -- but, yeah, I think when I needed to escalate I did.

ASTBURY: Okay.

The particular interview concluded there.

I'm moving on to our final interview in the summary bundles. This is an interview on 10 June, again, 2019, a little later in the day.

STONIER: Yes.

ASTBURY: Introductions and caution. Lucy Letby identified her personal diaries. She agreed that she would record various matters within shifts, personal thoughts, events, and some of the collapses. She confirmed only she ever wrote in it -- sorry, only she ever wrote in and had access to those diaries: My colleague asked you if you used your diaries, Lucy, to express your thoughts and feelings and you said sometimes. What would or explain to me what would trigger you to write that down?

STONIER: If there was something I was particularly struggling with or something that I felt I just needed to write down and express myself without telling anybody.

ASTBURY: Okay. And you said -- when he asked you the question, you said "sometimes". Can you quantify that? How often would you do that?

STONIER: So there have been points when it's been daily, when something's been difficult for me. Other times it might be weekly. I'm not sure.

ASTBURY: Right. And then my colleague asked you about the collapses of the babies and you said that you recorded those as well. Why?

STONIER: I think I've made reference but I don't know in what way I've recorded them, but...

ASTBURY: Okay. Can you explain that to me in more detail?

STONIER: I suppose it is just a way of me thinking things through myself in my own time and expressing those thoughts on paper.

ASTBURY: Okay. Explain to us what type of things you wrote, Lucy?

STONIER: I don't remember specifics but there have been times when I've really struggled and I thought maybe things were my fault and that people were blaming me, I've not been good enough, things like that, but I don't know that I've described -- that I've written down every collapse --

ASTBURY: Right, okay.

STONIER: -- or the detail of that collapse.

ASTBURY: Why would you want to reflect on those, Lucy?

STONIER: Because that's just how I cope with things. I don't talk to anyone about it, I just internalise things and do it in my own time. I think some of the diary entries I have made have been about how I feel about being potentially blamed for things, yeah.

ASTBURY: Okay. So do you remember when you started doing that, putting entries in diaries in respect of that?

STONIER: I think it was once I was removed from the unit.

ASTBURY: Okay. So we're looking, what, post-July 2016?

STONIER: Yeah, I think it was at a time when we were particularly busy and there were lots of staffing issues and I think I started to write things because I was starting to be used as second on call.

ASTBURY: What was the purpose of writing that down?

STONIER: I'm not too sure. I think it was just my own record of knowing of who I looked after and when, how many babies I have per shift.

ASTBURY: Is there no method at work to do that?

STONIER: Um, not unless you were -- not unless you went through each of the nursing notes. You'd have to look. There's no way of looking who looks after which baby on which days, no, without going into the nursing notes.

ASTBURY: Lucy Letby explained that the names appearing in the 2016 diary are those of the babies for which she was the designated nurse: Were there any concerns or issues on the unit at this time, Lucy?

STONIER: Yeah. There'd been mention about the concerns, that there had been a rise in mortality rate and we had staffing issues.

ASTBURY: This had been raised in February?

STONIER: I think it was early, yeah, I think so.

ASTBURY: Does that coincide as to why you have documented names?

STONIER: Yes.

ASTBURY: To what purpose?

STONIER: So I would know who I have looked after and how many babies.

ASTBURY: Okay. So you've also written things in red. Again, they're personal home points, are they?

STONIER: Yes.

ASTBURY: Lucy Letby was then shown a specific note from her diary, the exhibit reference KL4. Officer, that's the larger A4 sheet that was inside the diary; is that correct?

STONIER: Yes, that's correct.

ASTBURY: If you look to the bottom left, there's a -- highlighted in a box the words "Kill me".

STONIER: Mm.

ASTBURY: Why have you written that?

STONIER: Because I wish sometimes that I was dead and someone would just kill me.

ASTBURY: Why is that, Lucy?

STONIER: Because at that point I had lost everything and wasn't working on unit and was being -- I didn't really know what was going on and I hated working in the office.

ASTBURY: There's another box there, this box here, where there's a bit written in and then crossed out. Do you know what that says?

STONIER: No.

ASTBURY: So you don't remember when you did this?

STONIER: No.

ASTBURY: Because you didn't date or time it?

STONIER: No.

ASTBURY: Do you think you might have done it at work?

STONIER: I think -- looking at it, it started off as some notes about work and then I've just used it then as a doodle thing and added more to it.

ASTBURY: Then it's your way to express yourself, is that what you're doing?

STONIER: Yeah.

ASTBURY: I mean, would you put things that weren't sort of accurate or truthful?

STONIER: Well, I am not sure. Some of it is just doodling, it's something that comes in my mind at that time.

ASTBURY: Why have you kept this piece of paper, Lucy?

STONIER: I am not sure. I think I -- it was obviously put inside my diary and then just left there.

ASTBURY: But that suggests that it was -- to you, that suggests it was written around the time that you were using the diary.

STONIER: Yes, yeah, and I would say -- because it's some of this is relating to the work that I was doing in the office.

It's from when I was removed onwards.

ASTBURY: Okay. Thank you for that, Lucy. We have come to the conclusion of this particular interview now. Is there anything else you want to ask or tells us about the diaries?

STONIER: No, thank you.

ASTBURY: How are you feeling now?

STONIER: Well, I'm just a bit exhausted now.

ASTBURY: You feel exhausted? Okay. We'll turn this -- we'll complete this interview now anyway.

And we're given the time and the interview concludes?

STONIER: Yes, that's correct.

ASTBURY: Thank you. That's the conclusion of our summaries?

STONIER: Yes.

MR ASTBURY: I have no more questions. If you could wait there, please.

MR JUSTICE GOSS: Have you got many questions?

MR MYERS: No, I think perhaps, my Lord, about 10 minutes or so.

MR JUSTICE GOSS: All right. We are scheduled for a break.

MR MYERS: It may be, unless of course your Lordship or the jury wish for a break immediately, that I could conclude the questions for Sergeant Stonier now and then her evidence is completed insofar as we understand.

MR JUSTICE GOSS: All right. We'll do that.

Cross-examination by MR MYERS

MR MYERS: Sergeant Stonier, I just want to ask you a little bit about the process that we're dealing with here, the interview process, and how it applied in this case.

We're going to receive some agreed facts that give us dates and timings, so I am not expecting you to recall everything or us all to remember it all, but we know, and could you confirm, Ms Letby was arrested on three occasions?

STONIER: Yes, that's correct.

MYERS: The first was 3 July 2018?

STONIER: Yes.

MYERS: The second was 10 June 2019?

STONIER: Yes.

MYERS: And the third was 10 November 2020?

STONIER: Yes, that's right.

MYERS: And on each occasion that she had been arrested, she was interviewed whilst held in police custody?

STONIER: Yes, that's correct.

MYERS: So the interviews that we've been through are the interviews that took place after those arrests?

STONIER: Yes, that's right.

MYERS: Now over the period that she was interviewed and held in police custody she'd remain at the police station; is that right?

STONIER: Yes, that's right.

MYERS: And when someone is in that position -- this isn't personal to, Ms Letby, it is just the procedure -- their possessions are removed from them, their personal possessions, things like that?

STONIER: Yes, they are, when you first arrive at the custody suite --

MYERS: And they are logged and when they leave they are given back, aren't they?

STONIER: Yes, that's correct.

MYERS: And save for those times when they come to the room to be interviewed or go to the lavatory, they're kept in a cell? That's just what happens.

STONIER: Yes.

MYERS: And they're told they have various rights whilst they're in police custody; that's right, isn't it?

STONIER: Yes, that's correct.

MYERS: One of those rights is that they can have a legal representative if they want?

STONIER: Yes, that's right.

MYERS: And we know that Ms Letby had a legal representative with her during these interviews.

STONIER: Yes, she did, throughout them all.

MYERS: What happens with the legal representative, and what happened here, I'm just going to ask you to confirm, is when the representative came, he would speak to the police and be given what's called some advance disclosure; is that correct?

STONIER: Yes, that's right.

MYERS: And advance disclosure in this case meant that he received documents -- certain documents that related to the events that you were looking at and you were going to ask questions about?

STONIER: Yes, that's correct.

MYERS: In the case of each child that Ms Letby was going to be questioned about, she and her solicitor were provided with

documents, like nursing notes, where she'd made them -- this is before the interview?

STONIER: Yes.

MYERS: And also some of the key charts that we've been looking at in this trial; is that right?

STONIER: Yes, that's right.

MYERS: So things like the feeding chart or the observation chart for that child?

STONIER: Yes, that's correct.

MYERS: The solicitor would have the opportunity to speak to the police about what was taking place --

STONIER: Yes.

MYERS: -- whilst at the station and then also Ms Letby and her solicitor had time to speak about the interview that was coming up before it was held each time; is that correct?

STONIER: Yes, that's right. They were afforded the opportunity to speak in confidence.

MYERS: Then she would be able to go into the interview and answer questions if she chose to do so?

STONIER: Yes.

MYERS: And of course one of the rights that everybody has is not to answer questions if they wish not to do so?

STONIER: Yes.

MYERS: And they're reminded of that at the start of each interview?

STONIER: Yes.

MR JUSTICE GOSS: And also the consequence of not answering.

MR MYERS: And the consequence, which is that it could be, putting it in loose terms, potentially held against them if they don't answer questions.

STONIER: Yes, that's right.

MYERS: Thank you, my Lord.

Just so that we understand the scale of this, the documents that Ms Letby received wouldn't amount to the type of, I don't say this critically by the way, but it's not the suite of documents and collection of evidence we have on the sequence of events, things like that, was it?

STONIER: No, as I recall, as you've pointed out, it was the collection of nursing notes, feeding charts, ITU observation charts where Ms Letby had made reference or documented herself personally.

MYERS: Yes. So this is just an illustration, but if we just go to the [Baby O] divider, also [Baby O], which is in the second interview bundle, just behind the first divider, page 2, please, Sergeant Stonier, ladies and gentlemen.

MR JUSTICE GOSS: About halfway through this. The tab is [document redacted].

MR MYERS: It's the first of the [document redacted] tabs, just page 2, just to illustrate something if we could. It's the first tab it's the tab with [Baby O] actually written on it.

And if you look behind there in red it's got at the top corner "[document redacted]".

STONIER: Yes.

(Pause)

MYERS: Next to the tape counter that says 0203, having been asked about [Baby O], Ms Letby then goes through in some detail there about how she remembers him, who she was caring for and aspects of his care in that long paragraph, doesn't she?

STONIER: Yes, that's correct.

MYERS: So we can follow, that takes place after she's received notes that she has made and relevant documents relating to [Baby O], doesn't it?

STONIER: Yes.

MYERS: So this is after she's had the time to review that and speak about it?

STONIER: Yes, that's correct.

MYERS: You were involved in a number of the recordings, weren't you?

STONIER: Yes, I was.

MYERS: We're going to see, and this is just so I can deal with this now, that there are about 13 interviews or 13 tapes after the first arrest?

STONIER: Yes.

MYERS: That's the arrest on 3 July. There were 14 interviews held after the second arrest on 10 June. And there were three interviews after the third arrest on 10 November?

STONIER: Yes, that's correct.

MYERS: Right.

MR JUSTICE GOSS: Forgive me, Mr Myers, I don't want to interrupt, but can I just be clear, you've used tapes and interviews.

MR MYERS: I'll be quite clear. The figures I've given -- when I say 13 interviews, I'm referring to 13 sessions with the tape running.

STONIER: Yes.

MYERS: So it could be regarded as one interview process over the whole of her period of arrest, but we're going to see there are 13 separate tapes recording interviews, is that correct --

STONIER: Yes, separate recordings.

MYERS: -- between the 3rd and 5 July after the first arrest --

STONIER: Yes.

MYERS: -- and 14 after the second arrest on the 10th, and three of those after the third arrest on 10 November 2020?

STONIER: Yes, I believe so.

MYERS: Yes. In each case, whichever babies Ms Letby was going to be asked about, there would be some disclosure about them in advance of that tape or that interview taking place?

STONIER: Yes, at the start of that recording there would be.

MYERS: So we've got a series of opportunities to have memories jogged and then answer questions?

STONIER: Yes.

MYERS: In some of the interviews, the focus was on just one baby; is that correct?

STONIER: Yes, it depends how long that particular interview took in relation to that baby as to did we then move on to another baby or finish that interview.

MYERS: In some of interviews a number of the babies might be dealt with one after the other in the course of that one interview, or interview tape rather, that we are listening to?

STONIER: Yes, within that same recording.

MYERS: As the jury have been told, what we have here is that those interviews have been split up so they can be organised according to the children?

STONIER: Yes. As you say, a number of babies were covered within one recording on occasions.

MYERS: I'm going to just illustrate that if I could -- I'm coming to the end of what I want to ask you, by the way, bearing in mind the 10 minutes. I want to ask you to illustrate that with one of the interviews and the interviews towards the end of this process with [Baby N].

STONIER: Okay.

MYERS: We're going to look at some interviews in both folders, ladies and gentlemen, but the first reference I'm going to is in the folder that you should have open already, folder 2, but behind the [Baby N] tab, so [document redacted] and we're going to go to tab 3 for [document] redacted] and if we go to the first page behind tab 3, which will have at the top right-hand corner "[document redacted]". Let's just check we've all got that.

If we just look at the information on the front of that page, can you see Sergeant Stonier it's got the time of the interview? It says, about five or six of the lines down:

"Time: 10 November 2020. Time: 21.05 to 21.22."

STONIER: Yes.

MYERS: That relates to the time of the interview when it was dealing with [Baby N], doesn't it?

STONIER: Yes, that's correct.

MYERS: We can see, from the way it's been done here, that the actual interview commenced at 20.27 and finished at 21.22?

STONIER: Yes, that's correct.

MYERS: And we can see this is the interview, an interview on 10 November 2020, just above that?

STONIER: Yes.

MYERS: Which of course is the third occasion that Ms Letby had been arrested and then interviewed over the period of this part of the investigation?

STONIER: Yes.

MYERS: I say that because she had already been interviewed about [Baby N] on earlier occasions after earlier arrests, hadn't she?

STONIER: Yes, she had.

MYERS: If we just go towards the end of that interview, page 32, [document redacted], towards the bottom of the page we can see, as the questioning continues:

"I think I might need to stop now, please."

Can you see that?

STONIER: Yes, I can, yes.

MYERS: And then, over the page, the solicitor confirmed at page 33, "You want to stop?"

STONIER: Yes.

MYERS: And Ms Letby says, "Okay".

STONIER: Yes.

MYERS: And that, in fact, concluded the interviews that day, didn't it?

STONIER: For that day, yes, it did.

MYERS: As it happens, that day, Ms Letby had been interviewed about a large number of the babies we're dealing with in this case, hadn't she?

STONIER: Yes, she had, yes.

MYERS: And I'm just going to show that -- and again this isn't done as a criticism of the process, it's just so we understand what she was dealing with.

STONIER: Yes, of course.

MYERS: I'm going to go to file 1, ladies and gentlemen, if I could. We'll get the hang of what I'm doing fairly rapidly once we start, but if we go to the [Baby A] tab, it's the first tab in file 1 and go behind tab 3 for [Baby A]. So the page says [document redacted]. Have you got that, Sergeant Stonier?

STONIER: Yes, I have.

MYERS: This is 10 November, you were one of the interviewing officers. This part of the interview ran from 15.56 to 16.14?

STONIER: Yes.

MYERS: Can you see that? It was part of an interview that, as a whole, ran from 15.56 to 17.38, if we look below that.

STONIER: Yes, that's correct.

MYERS: So this is the first interview held on that day, isn't it?

STONIER: Yes.

MYERS: It dealt with [Baby A]. Then if we move behind tab 3 for [Baby B], [document redacted], it's tab 3 behind [Baby B], we can see there, during that interview, the next child to be dealt with was [Baby B]; is that correct?

STONIER: Yes, that's correct.

MYERS: If we move forwards to [document redacted], please.

Next was [Baby C]; is that right?

STONIER: Yes.

MYERS: He was dealt with after that?

STONIER: Yes.

MYERS: Then we go to [Baby E], tab 3, please, not [Baby D]. On the first page behind tab 3 for [Baby E], we can see that she then was asked questions about [Baby E] during the same interview; is that correct?

STONIER: Yes, that's correct.

MYERS: And in fact we don't need to repeat the process for all of these, I can just summarise it. In this interview between 15.56 and 17.38 you and your colleague went on to question Ms Letby about [Baby F] and [Baby G].

We can confirm it if you like but --

STONIER: It's okay, yes.

MYERS: That was between 16.45 and -- 15.56 and 17.38. A second interview took place that evening, didn't it?

STONIER: Yes.

MYERS: That's the one which we've looked at which concluded with [Baby N], didn't it?

STONIER: Yes.

MYERS: And that took place between 20.26 and 21.22. And if we were to repeat the same process, we'd find that the babies that Ms Letby was asked about then were [Baby H] -- do you want to confirm it, sergeant?

STONIER: It's okay, I can see.

MYERS: [Baby H]; that's correct, isn't it?

STONIER: It is, yes.

MYERS: [Baby I]?

STONIER: Yes.

MYERS: [Baby J], [Baby K]?

STONIER: Yes.

MYERS: [Baby L]?

STONIER: Yes.

MYERS: [Baby M]?

STONIER: Yes.

MYERS: And then finally [Baby N]?

STONIER: Yes, that's correct.

MYERS: And it was at the end of that she said she was tired and would like a rest?

STONIER: That's right.

MYERS: The questioning finished then and you moved to the remaining babies the following morning, didn't you?

STONIER: Yes, we did.

MYERS: You didn't need to return to [Baby N] though?

STONIER: No.

MYERS: Those were what might be regarded as the concluding interviews in the investigation?

STONIER: Yes, they were.

MYERS: So you covered quite a number of the babies in each interview, didn't you?

STONIER: Yes, we did.

MYERS: In the earlier interviews there might be one or several babies but not usually at that rate, would that be fair to say, in the earlier parts of investigation?

STONIER: Yes, they were more in-depth interviews with going through the notes (overspeaking) --

MR MYERS: That's right. All right.

Thank you very much Sergeant Stonier.

Re-examination by MR ASTBURY

MR ASTBURY: Only a few questions, my Lord.

Officer, you were asked about the -- a detained person's property being taken from them in the police station. Why does that happen?

STONIER: To protect both themselves and ourselves as interviewing officers. The same happens to every person that's brought into police custody: all their personal property is removed from them, documented on the custody record, and then returned to them once they leave the station.

ASTBURY: The custody record, just very briefly, what's the custody record, please?

STONIER: The custody record is a document that's completed by the custody sergeant, through a booking-in process, with the detained/arrested person where everything is documented, so their rights, their property, their medical health, it documents the time and date of arrest.

ASTBURY: Is anyone in particular responsible for the welfare of a detained person whilst in a police station?

STONIER: Yes, the custody sergeant is responsible.

ASTBURY: Right. Whilst in custody, does the custody sergeant have to ensure the treatment of that detained person is in accordance with the rules and regulations?

STONIER: Yes, they do, which is set out by PACE.

ASTBURY: Does that include a period of rest when required?

STONIER: Yes, it does.

ASTBURY: A night's sleep?

STONIER: Yes.

ASTBURY: Refreshments when required?

STONIER: Yes, we obviously take that on board too during the interview process.

ASTBURY: And meals at recognised mealtimes?

STONIER: Yes.

ASTBURY: All right. Now one of the rights you have also been asked about is a right to have a solicitor for the purposes of legal advice; is that correct?

STONIER: Yes. That entitlement is afforded to everybody who's arrested.

ASTBURY: Could you confirm, please, that it was the same solicitor who attended on all three of the dates that we've heard about?

STONIER: Yes, that's correct.

ASTBURY: That solicitor was present through each and every one of the interviews that we've heard about?

STONIER: Yes, he was.

ASTBURY: Same person, continuity; is that right?

STONIER: Yes, that's correct.

ASTBURY: We heard that that solicitor would be given disclosure before an interview took place.

STONIER: Yes, he was.

ASTBURY: And would they be then given the time to sit with their client, whoever it would be, and give them suitable advice based on that disclosure?

STONIER: Yes. As I say, they were afforded as much time as they required.

ASTBURY: Does the solicitor's responsibility continue during the course of the interview?

STONIER: Yes, throughout the whole interview process.

ASTBURY: Are they at liberty to interject if they feel appropriate?

STONIER: Yes, at any point.

ASTBURY: Did we see, on a number of occasions, Ms Letby's solicitor interjecting during the course of these interviews?

STONIER: Yes, he did on a couple of occasions.

ASTBURY: And if, for example, a solicitor were to say, please may we have a break because my client is tired, what would your reaction be to that?

STONIER: Yes, absolutely, and I think that did happen on an occasion where a break was afforded to Ms Letby.

ASTBURY: Thank you. And we've heard about the recording of the interview.

STONIER: Yes.

ASTBURY: Was the interview recorded both by audio recording --

STONIER: Yes, it was.

ASTBURY: -- but also a video recording?

STONIER: Yes, it was. So all interviews have been video recorded as well.

ASTBURY: And they are available if any issue arises as a consequence?

STONIER: Yes, they are.

MR ASTBURY: I have no more questions. Does my Lord have any?

Questions from THE JUDGE

MR JUSTICE GOSS: PACE. You referred to PACE, the Police and Criminal Evidence Act, which is the governing statute under which there are codes of practice that have to be followed?

STONIER: Yes.

MR JUSTICE GOSS: And one other thing by way of clarity: once arrested, a person can only be detained in police custody for a certain period of time. Applications can be made to extend that period of time to the court.

STONIER: Yes, that's correct.

MR JUSTICE GOSS: And then they have to be released or charged?

STONIER: Yes, that's right.

MR JUSTICE GOSS: I'm sure no one wants to -- if the jury have televisions, which I anticipate they do, they've probably seen programmes that have people taken into police custody, which is actually filmed in a police station generally. It's not a drama that's created, so it is actually what takes place.

MR ASTBURY: Yes.

MR JUSTICE GOSS: All right. Thank you very much.

Thank you very much indeed, Sergeant Stonier, that completes your evidence. Thank you for coming.

It's later than anticipated, but we will have still a break though. A ten-minute break, members of the jury.

(12.18 pm)

(A short break)

(12.30 pm)

MR JUSTICE GOSS: Mr Johnson.

MR JOHNSON: My Lord, Eirian Powell, please.

MS EIRIAN POWELL (recalled)

Examination-in-chief by MR JOHNSON

MR JOHNSON: Welcome back, Mrs Powell. I think you were last with us on 14 December last year, where you gave the jury some evidence about [Baby G].

POWELL: Yes.

MR JOHNSON: Would you wait there, please, because I understand there are some more questions for you.

Cross-examination by MR MYERS

MR MYERS: Mrs Powell, I just have some questions for you, general questions relating to some of the matters we're looking at, not about any particular baby, but just on the unit. If you could help me with this, I'd be grateful.

Just to remind you, at the time we're looking at in 2015 through to 2016, were you the ward manager on the neonatal ward at the Countess of Chester?

POWELL: I was.

MYERS: Was that a position that you held between 2011 and the end of 2017?

POWELL: It was.

MYERS: So in fact you would have been a ward manager over the time that Ms Letby was working as a nurse on the neonatal unit itself?

POWELL: I was, yes.

MYERS: What stage of her career was she at when you first met Lucy Letby?

POWELL: That was before -- well, I think it was before 2011, I'm not sure of the exact time, as she was a student at that time.

MYERS: So you met her when she was a student; is that correct?

POWELL: Yes.

MYERS: Had she come to the neonatal unit as part of a four-week placement?

POWELL: She was indeed, yes.

MYERS: And that was her nursing training?

POWELL: Yes.

MYERS: And that was at the University of Chester, wasn't it?

POWELL: It was indeed.

MYERS: Was she somebody who, so far as you could assess at that time, was striving very hard to achieve good standards as a nurse?

POWELL: She was indeed, yes.

MYERS: And seemed to be very keen to improve her practice?

POWELL: Yes, she did strive to -- to get where she wanted to be.

MYERS: And you were able to see her from that period onwards to the point that that she was working on the unit; is that correct?

POWELL: Yes.

MYERS: And would it be fair to say that in the time you saw her working there, on the unit, she always struck you as an exceptionally good nurse?

POWELL: Yes, she was.

MYERS: We know now from the case that there's different bands or level of nurse on the unit.

POWELL: Yes.

MYERS: For a nurse to be able to look after intensive care babies, does he or she have to become specifically qualified for that?

POWELL: She does, yes.

MYERS: Right. Was that something that Ms Letby did in due course?

POWELL: She did. She did a few sessions in Liverpool Women's Hospital. She did her preceptorship with us on the unit, which is -- because it's a specialty, it needs to be a twelve-month preceptorship. And then she went on to Liverpool then to do an induction programme, which was a ten-week placement.

MYERS: Do you know in the period we're looking at, say from 2010 to 2015, roughly when she would have done that ten-week placement?

POWELL: It's usually done within 12 months that they'd been on the unit because sometimes they're not suited, they don't know that that's not for them, so they move on, and therefore the investment in doing the induction programme wouldn't be worthwhile. So they need to show that they've got a keen interest before they go on these programmes. So they have the preceptorship, they actually pass them through to go on the induction programme, and then that will have taken them to the first 12/18 months' time on the unit.

MYERS: Right. And through that period, she was, so far as you could see, committed to what she was doing?

POWELL: She certainly was, yes.

MYERS: And she wanted to develop in her progress as a nurse; is that correct?

POWELL: Yes, she was.

MYERS: Did she eventually go on what we've heard is the qualified in specialisation course, QIS?

POWELL: She did.

MYERS: Is that what qualifies a nurse so that he or she can then look after the intensive care babies?

POWELL: It is.

MYERS: And they're the most poorly babies on the unit?

POWELL: That's right.

MYERS: And was that something she qualified in during 2015?

POWELL: I can't be sure of the timeline.

MYERS: But that, again, is a particular course that she had to go on; is that right?

POWELL: She has to, yes.

MYERS: And was that at Liverpool Women's Hospital --

POWELL: Again, yes.

MYERS: -- as well? And Liverpool Women's Hospital, is that what's called a tertiary unit?

POWELL: It's a level 3.

MYERS: A level 3?

POWELL: Yes.

MYERS: So that deals with the most intensive, prolonged level of care for babies?

POWELL: And the most premature.

MYERS: And the most premature?

POWELL: Yes.

MYERS: In terms of the work Ms Letby did, and I'm looking particularly at the period we're looking at in this case, 2015 into 2016, is she one of the nurses who, as it happens, did do a lot of the work with the intensive care babies on the neonatal unit?

POWELL: I can't remember exactly for that time. She must have done 11, 12, 13 -- yes, she would have done, yes.

MYERS: Because she'd got her QIS qualification --

POWELL: She was (sic), yes.

MYERS: And did she strike you as somebody who was very hard-working and flexible in terms of shifts?

POWELL: Yes, she was. Extraordinarily so.

MYERS: And that meant, I'm going to suggest, and ask you if this is right, that she ended up looking after the intensive care aspect of the babies very often?

POWELL: Yes, she did, and certainly there was a swing sometimes between the intensive and high dependency and vice versa.

MYERS: Yes.

POWELL: Because even though they step down to high dependency, they can as easily become high -- um, intensive care, you know, until they stabilise enough --

MYERS: Yes.

POWELL: -- to actually become special care.

MYERS: And throughout that period, from what you could see, her standards remained as high as --

POWELL: Yes.

MYERS: -- you could have hoped for?

POWELL: Yes.

MYERS: And so did her commitment?

POWELL: It was indeed. As I have mentioned, she was very particular and -- attention to detail.

MYERS: We know that in or around April 2016, Ms Letby was moved in general to day shifts.

POWELL: Yes.

MYERS: We see, in fact, she also did cover night shifts from time to time, but most of the shifts were day shifts; is that right?

POWELL: Yes, it was.

MYERS: Now, that was after a time when there had been a number of deaths on the unit, hadn't there?

POWELL: There was.

MYERS: And Ms Letby had been identified as someone who'd been on duty and present at a number of those deaths, hadn't she?

POWELL: She had.

MYERS: So that we all understand the reason for the shift at that point, was the purpose of that to give her some more support by putting her on the day shifts?

POWELL: Yes.

MYERS: It wasn't meant to be a punishment of some sort?

POWELL: No.

MYERS: And why was going on to the day shifts something which would give her more support? What was the difference?

POWELL: Well, because there were more people about to be able to support her. There were the opportunities for debriefs with the consultants and the other doctors to sort of help at that time. There were also debriefs 10 days later. There were opportunities for some HR support, occupational support, you know. So there was, in the daytime, better opportunities for her.

MYERS: That sort of support?

POWELL: That's right.

MYERS: But, as it happens, the unit remained busy, didn't it --

POWELL: It did.

MYERS: -- in terms of babies?

POWELL: Yes.

MYERS: And she was still required from time to time to work at night, as it happens, wasn't she?

POWELL: I believe so, but I'm -- I can't be specific.

MYERS: I'm not going to ask you to recall particular shifts, Mrs Powell. Thank you.

Moving on from there --

POWELL: Yes.

MYERS: -- we know Ms Letby was moved to a non-clinical role --

POWELL: Yes.

MYERS: -- in early July 2016?

POWELL: Yes.

MYERS: I'm going to ask you some questions about that.

POWELL: Okay.

MYERS: That was a role working in a different part of the hospital, in an office-based role; is that correct?

POWELL: It was.

MYERS: And did that happen round about the time of her return from annual leave in 2016? Do you recall that?

POWELL: I don't recall.

MYERS: We've got a couple of emails. I'm going to show you the first one. Tab 226, please, Mr Murphy. We're going to see it says from Yvonne Griffiths, but if we just go right down to the bottom of this, can see it's:

"Kindest regards, Eirian Lloyd Powell."

POWELL: Yes.

MYERS: Which is you, isn't it? This is Friday, 15 July 2016 at 11.16. I'll read it through, if I may, but with your name at the end of it, do we presume you must have sent this out and under the Yvonne Griffiths email?

POWELL: No, Yvonne would have sent it out under my email.

MYERS: Under yours?

POWELL: Yes.

MYERS: Can I read it to see if you're familiar with it:

"Hi everyone. In preparation for the external review, it has been decided that all members of staff need to undertake a period of clinical supervision. Due to our staffing issues, it has been difficult to determine how we undertake this process. We can only support one member of staff at a time, therefore we have decided that it would be useful to commence with staff who have been involved in many of the acute events, facilitating a supportive role to each individual.

"Therefore Lucy has agreed to undergo this supervision first, commencing on Monday, 18 July 2016. I appreciate that this process may be an added stress factor in an already emotive environment, but we need to ensure that we can assure a safe environment in addition to safeguarding not only our babies but our staff. This is not meant to be a blame or a competency issue, but a way forward to ensure that our practice is safe. It will probably be developed into a competence-based programme to be undertaken every 2 to 3 years in line with our mandatory update training."

It's signed off in your name, but you recall, or you believe, by Yvonne Griffiths?

POWELL: That could have been me doing it and asking her to check it over, yes.

MYERS: Right. Now, this coincides with a period, roughly, that Lucy Letby was taken off the unit and put on non-clinical duties, doesn't it?

POWELL: Yes.

MYERS: And, in fact, was it explained to Ms Letby, certainly at the outset, that competencies would be reviewed across the staff generally and she would be the first?

POWELL: Yes.

MYERS: She did actually become quite upset at being removed, didn't she?

POWELL: She did.

MYERS: And as it appeared that competencies were in question she became more upset, didn't she?

POWELL: I don't recall the exact timing. I just think it was upsetting that she was being removed.

MYERS: Yes. Was she in due course told that her competencies would be reviewed or tested?

POWELL: No, it's just that she had to go through the competencies to come back on the unit.

MYERS: That's what she was told?

POWELL: Yes.

MYERS: Now, in fact, was this something which, in reality, was taking place only with Lucy Letby or was it --

POWELL: At that time, yes.

MYERS: At that time.

POWELL: Yes.

MYERS: And was it something which took place also because there were doctors on the unit who wanted her to be removed from it as well? Was that part of what lay behind this?

POWELL: Um... At that time -- what time are we discussing, July?

MYERS: This is July 2016, yes.

POWELL: So that was after June. Yes, I believe that one -- yeah.

MYERS: I'm not going to ask for specific details, but just to keep pace with where we are with everything.

POWELL: Right.

MYERS: Now, as part of what was happening with Ms Letby was there a meeting that was held with Sian Williams, a lady called Sian Williams?

POWELL: Yes.

MYERS: And to assist everybody, Sian Williams was the Deputy Director of Nursing at the Countess of Chester.

POWELL: Yes.

MYERS: And that's a meeting I'm going to ask you about that took place again in early July 2016.

POWELL: Yes.

MYERS: Round about the time of --

POWELL: Yes.

MYERS: -- we're looking at now.

POWELL: I have got a timeline on my computer, so -- but I have no access to it. Not my computer, my work's computer.

MYERS: It's the period that we're looking at, so I think that will be all right. If we do need to look at it, we can.

There was a meeting that took place with Sian Williams at about this time.

POWELL: Yes.

MYERS: I'm going to suggest to you one of the things that happened was that Sian Williams told Lucy Letby not to talk to other staff members about what was taking place with reviewing her competencies. Do you recall anything like that?

POWELL: I don't recall that, no.

MYERS: Do you recall that Sian Williams wanted to create the impression that what was taking place with Lucy Letby was voluntary, although Lucy Letby didn't actually want to do this? Do you remember something like that taking place?

POWELL: Um... I remember the meeting was very upsetting and certainly for Lucy and myself.

MYERS: Yes.

POWELL: I can't remember the actual details. I know it was suggested that she needed to come off.

MYERS: Was she told that she wasn't to be talking about what was taking place with her with other members of staff?

POWELL: I don't recall that, but then I don't recall very much of that meeting --

MYERS: All right.

POWELL: -- other than we were both quite upset because we went to HR straight after that.

MYERS: Again, tell us if you can recall this or not, but I'm going to suggest that it was made plain that there were a couple of people who she got support from, who she could talk about these things with, but not with everybody. Do you recall something like that being said?

POWELL: No.

MYERS: Anything about Minna Lappalainen and [Nurse E] and [Dr A] being people she could have -- speak with about what was taking place?

POWELL: No, I don't. I don't remember it.

MYERS: But was Ms Letby upset --

POWELL: Yes.

MYERS: -- at what was taking place?

POWELL: Yes, very.

MYERS: And she didn't want --

POWELL: I remember that.

MYERS: -- to come off the unit and be treated --

POWELL: Well, I don't think --

MYERS: -- in this way?

POWELL: -- she had much choice because she was distraught at that point.

MYERS: And that's it. She didn't have much choice, did she?

She was being told what she was going to have to do?

POWELL: Yes.

MYERS: Yes, and that made her more upset as well, didn't it?

POWELL: Well, I think she was upset by the -- what was said in the meeting, you know.

MYERS: And was that that there was a problem with her practice?

POWELL: Not necessarily practice, but what was suggested.

MYERS: That she was responsible for things that had happened?

POWELL: Yes.

MYERS: Yes, and she was upset?

POWELL: Very.

MYERS: Can we go forwards to another email that was sent, please, Mrs Powell.

It's at tile 263, Mr Murphy, on the post-indictment schedule.

Again, this is an email that says "from Eirian Powell" and it's dated --

POWELL: And that would be -- yeah.

MYERS: That's you?

POWELL: Yes.

MYERS: Tuesday, 9 August 2016 at 14.19 hours:

"Dear all. There are currently opportunities for staff to apply for secondments throughout the trust. It has therefore come at an opportune time for us and we were able to facilitate this for

Lucy. Lucy is currently seconded to the Risk and Patient Safety Office for a period of 3 months. Laura is currently seconded to the haemodialysis unit and will be returning in November 2016. Should anyone have an interest in other areas, please discuss this further during your appraisal or come to me directly."

POWELL: Yes.

MYERS: We can see we've moved forward to August 2016 at this point. The email talks about this opportunity coming at an opportune time and it was possible to facilitate this for Lucy. Do you see that?

POWELL: Yes.

MYERS: Was the reality in fact that it wasn't really something she had picked to do, it was something she was being compelled to do, wasn't it?

POWELL: Yes.

MYERS: And that was something that upset her as well, wasn't it?

POWELL: I don't know whether she was upset about this email, sorry.

MYERS: She was upset during this period?

POWELL: Yes.

MYERS: And increasingly so as she learnt some of the things that were being said about her; is that right?

POWELL: Yes.

MYERS: And the kind of allegations that were being made?

POWELL: Okay, yes.

MYERS: Do you agree with that?

POWELL: Yes, I agree.

MYERS: Do you recall whether anybody else was taken to have their competencies reviewed or looked at again in the way Lucy was or is it something that only happened to Lucy Letby?

POWELL: Well, it was because in the midst of all that was going on at that moment and everybody has their competencies reviewed.

MYERS: Yes, all right.

POWELL: But not to that degree, because we were trying to get Lucy back on the unit, so we had to try and prove that the competency issue wasn't the problem.

MYERS: And not with those sort of things being said about them by other people?

POWELL: No.

MR MYERS: All right. Thank you, Mrs Powell. That's what I wanted to deal with.

Re-examination by MR JOHNSON

MR JOHNSON: Just two issues I'd like to ask you about, Mrs Powell, and I just want to accurately remind you of a couple of the things that you've just said, first of all.

First of all, you were -- do you remember at the beginning of the evidence you were being asked about Lucy Letby's training and her commitment?

POWELL: Yes.

JOHNSON: You were asked this question:

"And throughout that period, from what you could see, her standards remained as high as --"

And you said "yes".

"-- you could have hoped for", said Mr Myers.

And you said "yes".

Then this was said to you:

"And so did her commitment?"

And you replied:

"It was indeed. As I have mentioned, she was very particular -- and attention to detail."

Was she a very competent nurse?

POWELL: Yes, she was.

JOHNSON: Did she make mistakes?

POWELL: Like everybody makes mistakes, and she was very good at reporting her mistakes as well as her colleagues' and indeed her friends'. It made no difference: a mistake was a mistake, no matter how small it was. She was very good to relay them.

MR JUSTICE GOSS: Carry on.

MR MYERS: You said something about reporting her colleagues and her friends as well.

POWELL: Yes.

MR JUSTICE GOSS: I was just going to clarify that.

So she would report any mistake that she made?

POWELL: Yes.

MR JUSTICE GOSS: And she would report any mistake that any other --

POWELL: Yes.

MR JUSTICE GOSS: -- nurse practitioner made?

POWELL: Yes, irrespective of the seniority or whatever, it was an error, and she would also ensure that she would see me, when I'd come on, to explain what had happened.

MR JUSTICE GOSS: What about medical staff?

POWELL: Yes, it wouldn't matter.

MR JUSTICE GOSS: It didn't matter? Nurse, doctor, she'd report them?

POWELL: Yes, it didn't matter.

MR JOHNSON: Later in the questioning, you were asked about Lucy Letby being upset at being moved. Do you remember that series of questions?

POWELL: Yes.

JOHNSON: You said:

"I remember that. I don't think she had much choice because she was distraught at that point."

POWELL: She was.

JOHNSON: And it was said to you:

"Question: She didn't have much choice, did she?

"Answer: Yes.

"Question: That made her more upset as well, didn't it?

"Answer: "I think she was upset by the -- what was said in the meeting."

POWELL: That we were in.

JOHNSON: Yes.

POWELL: Yes.

JOHNSON: What was said at the meeting?

POWELL: Well, that she would have to come off the unit and I just -- honestly, I cannot remember what Sian actually said.

JOHNSON: The next question that was put to you:

"Question: "And was that that there was a problem with her practice?"

"Answer: Not necessarily practice, but what was suggested."

Then Mr Myers said to you:

"Question: That she was responsible for things that had happened?"

And you agreed with that. What was being suggested?

POWELL: Well, that she was the predominant -- no, she was the commonality within all the deaths that were there.

That's all I could say.

JOHNSON: When you agreed with what was put to you by Mr Myers, that she was responsible for things that had happened, and you said yes, what was being suggested?

POWELL: Well, there was nothing suggested.

JOHNSON: So you should have answered, what, "no" to that question?

POWELL: Okay.

JOHNSON: Well, I don't know. I'm asking you.

POWELL: I don't know. It was just that that was the decisions that the heads had made.

JOHNSON: What was it that was upsetting her, Mrs Powell?

POWELL: That she thought that she'd caused the deaths of the children that were involved, that were in the report that I'd actually compiled.

MR JOHNSON: Thank you. Does your Lordship have any questions?

MR JUSTICE GOSS: No, I don't, thank you very much.

Thank you, Mrs Powell, for coming back and giving evidence again. That completes your evidence and you're free to go. Thank you.

(The witness withdrew)

MR JOHNSON: My Lord, I'm going to ask for a slightly extended break, please, so we can resolve a few issues between us.

MR JUSTICE GOSS: Yes. Just so that we can have, if possible, some update as to timetable, because days are increasingly precious.

MR JOHNSON: Oh yes.

MR JUSTICE GOSS: They're always precious, but we've seen there are unavoidable circumstances which mean that we're going to have a bit of a sporadic run from now on.

Is it anticipated that the prosecution evidence will end today?

MR JOHNSON: Yes.

MR JUSTICE GOSS: Right. So we will reach that stage.

There you are. On that note, we will break off now.

How long would you like, Mr Johnson? Don't underestimate the time because there's nothing worse than everyone coming here expecting to start and being told, no, we're delayed and delayed.

MR JOHNSON: 2.30.

MR JUSTICE GOSS: Right. This is in relation to the outstanding evidence?

MR JOHNSON: So the jury knows what's coming, there's no secret, there's some agreed facts and it's literally dotting Is and crossing Ts.

MR JUSTICE GOSS: And checking them. So an hour and a half we're going to have until 2.30, please, ladies and gentlemen. When we do finish today, at whatever time that is, I will give you the revised non-sitting day list for you. Thank you very much.

(In the absence of the jury)

MR MYERS: Only a brief matter, thank you, my Lord. By way of the agreed facts, we anticipate we will have dealt with and cut through a good deal of evidence that would otherwise have come from the police officers. One officer who we had originally intended to have to give evidence was Detective Chief Inspector Hughes, who was the officer in the case originally. In fact, the issues that would have been dealt with in his case have been resolved one way or the other and there is one matter which will remain -- and having considered this with Mr Johnson, remain as a matter of comment, but I want to explain to your Lordship what it is so it doesn't create any surprise for your Lordship when we make it. It's only a small matter.

Your Lordship may recall it was put to Dr Evans by me at an early stage in his evidence that he would have heard about the suspicion of air embolus before he came to write his reports and he was keen to say that that isn't what happened, he got there independently.

It's a contention we make, given the nature of the investigation, that he will have heard about that at some point from someone, whether it's at the NCA or the police. He says he didn't. If Detective Chief Inspector Hughes gave evidence, it's a matter we would put to him, but the view taken -- and we understand this -- is in fact he can hardly account for who might have said what, where or when and there is no way of auditing that at all. It is something that could be said and the evidence from Dr Evans is he didn't say it and our contention is that --

MR JUSTICE GOSS: Unless it's said to him, to the detective chief inspector, that's the only way that he could give direct evidence of that.

MR MYERS: Yes. That would seem to be right. So rather than leaving it in that speculative way with him, it's a matter still that we maintain, it's a matter Dr Evans, in terms of evidence,

disagrees with, but it seemed appropriate to let your Lordship -
- it remained an issue of contention between us so that the
court didn't form the view that we were pursuing something
without having -- the prosecution know it's something we would
have otherwise put to the officer in the case.

MR JUSTICE GOSS: So what you're saying is you will still make
the comment --

MR MYERS: We will.

MR JUSTICE GOSS: -- and address the jury in due course in
relation to that aspect --

MR MYERS: Yes.

MR JUSTICE GOSS: -- but that issue has not deliberately been
avoided but the fact is (overspeaking) I understand entirely.

MR MYERS: We know -- we have the history of events so far as
they're in evidence and we will work with that. But insofar as
it can't be said that he can possibly account for who has said
what to who or where, it's an artificial exercise, just so your
Lordship isn't concerned by it being raised when it might not
appear to have been dealt with in evidence, I let your Lordship
know now.

MR JUSTICE GOSS: Thank you very much. That's helpful to know.

There is then -- there will come a point when we're going to
address the timetable. We may do that this afternoon then --

MR MYERS: Yes, of course.

MR JUSTICE GOSS: -- if that will be convenient when we've
completed the evidence for the prosecution. You can then raise
matters you want to raise.

MR MYERS: And it may be your Lordship may wish to deal with, I
know not, before the jury have departed, in case there's any
directions to give to them as to when they might be required
again. A matter for your Lordship.

MR JUSTICE GOSS: I will. Thank you very much. Good, thank you.
2.30, please.

(1.00 pm)

(The short adjournment)

(2.30 pm)

(In the absence of the jury)

MR MYERS: My Lord, we're grateful -- just before the jury come in, we're grateful for the time to finalise the admissions, which has been done. There's one matter which is agreed but will be introduced during the defence case, so that your Lordship is appraised of it, and it relates to additional Facebook searches by the defendant.

That's been reduced to a schedule with some preliminary points as to the nature of those searches.

That's all been agreed. There's one or two entries on the schedule to finalise. But the view has been taken, and we understand it, it's better introduced as part of the defence case, so it won't appear in the admissions, but it will be admitted at that stage. Thank you.

MR JUSTICE GOSS: I was anticipating that it would be introduced in some way or another because I was aware of it.

MR MYERS: We've reached agreement, just the final points on the schedule, but otherwise we'll wait for the defence case.

MR JUSTICE GOSS: Thank you very much. I did prepare an updated non-sitting days. As you'll see, I have put "As at 27/04".

MR MYERS: Yes, that's today's non-sitting dates.

MR JUSTICE GOSS: Yes, exactly. All right. Thank you very much. Jury, please.

(In the presence of the jury)

MR JUSTICE GOSS: I think you've been given the updated sheet. You'll see I've put "As at 27/04", today's date.

Right, thank you very much. So that's obviously for you to take with you when you leave later this afternoon.

Summary of agreed facts (read)

MR ASTBURY: My Lord, we are moving on to some more agreed facts. Could I ask for the documents to be distributed with the members of the jury.

(Handed)

We've had some agreed facts before, if I can remind everybody, behind divider 3 of jury bundle 1, and they follow sequentially.

My Lord, the admissions numbered 26 to 31 in sections 5 and 6 were read in fact before DC Johnson gave evidence. The jury will remember about the searches, so I don't think there's any reason to read them back into the record unless my Lord would wish me to do.

MR JUSTICE GOSS: No, I don't see any need for that. We can just put those in.

MR ASTBURY: These are just paper copies of what we heard on that particular day. I'm going to pick this up at section 7 if I may and read into the record then once everybody's ready.

Section 7 bears the heading "Interviews under caution and charge".

Number 32. Lucy Letby was interviewed under caution at the western custody suite Chester on the following dates between the following times.

It's represented in a table, my Lord, and it indicates: Interview 1. 3 July 2018, between 4.10 and 4.20 in the afternoon, 16.10 and 16.20. It contains the references should they become relevant.

Interview number 2. 3 July. 19.29 to 20.35.

Interview number 3. 4 July. 10.23 to 12.04 hours.

Interview 4. 4 July 2018. 13.41 to 14.17 hours.

Interview number 5. 4 July 2018. 18.54 to 20.08 hours.

Interview number 6 on that date, 4 July, 20.17 to 20.58.

The interviews then continued the next day: Interview number 7. 5 July 2018. 09.43 to 10.07.

Interview number 8. 5 July. 11.05 to 11.48.

Interview 9. 5 July 2018. 13.15 to 13.44.

Interview number 10. 5 July 2018. 14.25 to 14.49.

Interview number 11. 5 July 2018. 15.34 to 16.26.

Interview number 12. 5 July 2018. 18.05 to 19.14.

My Lord, the jury will notice there's an asterisk next to the reference. That will be explained in a moment.

Interview number 13. 5 July 2018. 20.27 to 20.34, described as a welfare interview.

Moving on: Interview 14. 10 June 2019. 12.24 to 13.39.

Interview 15. 10 June 2019. 14.41 to 16.14.

Interview 16. 10 June 2019. 18.13 to 19.29.

Interview 17. 10 June 2019. 20.00 hours to 21.03.

The following day, interview 18. 11 June 2019.

13.27 to 13.40 hours.

Interview 19. 11 June 2019. 14.16 to 14.58.

Interview 20. 11 June 2019. 17.44 to 18.36.

Interview 21. 11 June 2019. 19.22 to 20.39.

Interview 22. 11 June 2019. 21.13 to 21.39.

Interview 23. The following day, 12 June 2019.

09.40 to 10.15.

Interview 24. 12 June 2019 between 11.20 and 11.45 hours.

Interview 25. 12 June 2019. 13.36 to 14.00.

Interview 26. 12 June 2019. 15.55 to 16.10.

Then at the end of that particular day, interview 27, 12 June 2019, 16.35 to 16.40, a further welfare interview.

Moving on to the third date of arrest -- sorry, interview 28 on 10 November 2020, 15.56 to 17.38.

Interview 29. 10 November 2020. 20.26 to 21.22.

A double asterisk on this occasion, which we'll come to in a moment.

Finally, interview 30 on 11 November 2020 between 10.35 and 11.06 hours.

Admission number 34 or agreed fact number 34:

"At the commencement of each interview (save for after the breaks in the interviews marked star and double star above when it was not repeated), the defendant was cautioned in the following terms:

"You do not have to say anything but it may harm your defence if you fail to mention when questioned something that you later rely on in court. Anything you do say may be given in evidence."

That she, Ms Letby, was legally represented with her solicitor present throughout and the interviews were visually recorded.

Each and every interview was fully transcribed. The recordings and full transcripts are exhibited in this case.

Finally, agreed fact number 36. The summarised/edited transcripts presented during the trial are accurate reflections of the relevant parts of the above interviews agreed for presentation between, agreed that is, the prosecution and defence.

So that's that section, my Lord.

MR JUSTICE GOSS: Thank you.

MR ASTBURY: We'll move on to some more agreed facts if we may.

MR JUSTICE GOSS: Yes, certainly.

(Handed)

MR ASTBURY: If these can go in divider 3 behind the last set, please. There are some exhibits to show alongside these, so if we pause for a moment while we give Mr Murphy a chance to catch up.

(Pause)

It'll take 5 minutes, I'm sorry. I've got an index for the interviews to hand out, if that doesn't cause too much confusion, and we could to use the time --

MR JUSTICE GOSS: I think we can cope with that.

MR ASTBURY: Thank you, good.

My Lord, in respect of the two interview bundles, as requested, we've done an index.

MR JUSTICE GOSS: That will be helpful. Let's do that. You remember I thought it would be helpful to have for each of the

files an index as to where they come. So one for each, I anticipate. Is that right?

MR ASTBURY: Two sheets, one to go in each of the bundles.

(Handed)

(Pause)

MR JUSTICE GOSS: Whilst we're waiting, Mr Astbury, I have a recollection from many months ago, I mean the early part of the trial, when the jury were played video recordings of the neonatal unit and a question arose about when it was that those recordings were taken.

I know that a lot of witnesses were asked questions by reference to plans and some of the recordings. I think it was said that you were going to try and find out when the dates were. Does it come in this?

MR ASTBURY: It's in the document, we have remembered, I'm pleased to say. It's 3 October 2021 when we get there.

So it was a little late in the piece but that's when it was --

MR JUSTICE GOSS: I hadn't read on through these, but I just thought, whilst we were filling in time, before I forgot -- we have dealt with it?

MR ASTBURY: No, we have incorporated it. Thank you.

MR JUSTICE GOSS: It is a long time ago that we saw those.

MR ASTBURY: Yes. We're nearly there, I'm told. Thank you.

(Pause)

MR ASTBURY: There are a number of exhibits which are mentioned in the admissions, so we thought it best to have them available. I am very grateful to Mr --

MR JUSTICE GOSS: That's absolutely fine.

(Pause)

MR ASTBURY: We thank Mr Murphy for his efforts and we're ready to move on.

We have, hopefully behind divider 3, admission 37, which follows on from our earlier agreed facts, under the heading "Telecommunications" and the sub-heading "Telephone handset".

Number 37. Items seized from 41 Westbourne Road, Chester, in July of 2018, included the following communications device: an HTC One Mini 2 internet-enabled smartphone. The exhibit reference follows, JB31, and of course the date upon which it was seized, 04/07/18.

The digital contents of that exhibit, JB31, have been extracted and stored in a file entitled "JB31 04/07/18 device examination report". This extraction is the source of the relevant WhatsApp, SMS text and Facebook Messenger communications relied upon by the prosecution.

So all those messages, my Lord, we see in the sequence of events charts --

MR JUSTICE GOSS: Have come from that phone?

MR ASTBURY: -- come from that phone.

The images of a thank-you card from the [Babies E & F] family.

That exhibit reference is SGO300419-2. And two images in particular, 5300 and 5301, were recovered from the images file on the handset JB31.

I'm going to ask Mr Murphy if he can put up, please, J2462 and the following page, please.

Those two images found on the handset. My Lord, they also appear in the sequence of events chart for the [Babies E & F] family.

Agreed fact number 40. Further analysis of the metadata from these images establishes that they were taken on the same device, JB31, at 03.40 hours on 20 November 2015. The GPS coordinates indicate it was taken in a location in the south corner of the Women and Children's building at the Countess of Chester Hospital.

Can I ask Mr Murphy next, please, to go to J13163.

Agreed fact number 41 reads:

"The images of a sympathy card addressed to the [Baby I] family."

And the exhibit reference SGO300419-1 and the images 5292 and 5293 were recovered from the images file on the handset JB31.

If we can maybe look at the second page as well Mr Murphy, thank you.

MR JUSTICE GOSS: That's just an enlargement of the first page. Can we go back to the enlargement on the first page because, for my part, I couldn't read it in the smaller form, just to remind ourselves what it said.

MR ASTBURY: So it's a card addressed to:

"[Mother of Baby I], [Father of Baby I] and family. There are no words to make this time any easier. It was a real privilege to care for [Baby I] and get to know you as a family, a family who always put [Baby I] first and did everything possible for her. She will always be a part of your lives and we will never forget her. Thinking of you today and always. Sorry I cannot be there to say goodbye."

I think the second image was the other side of the interior of the card, signed "Lots of love, Lucy".

MR JUSTICE GOSS: And that was the day of the funeral, which she couldn't attend?

MR ASTBURY: Yes.

Reading on to agreed fact number 42:

"Further analysis of the metadata from these images establishes that they were taken on the same device, [that being JB31] at 07.34 hours on 10 November 2015. The GPS coordinates indicate they were taken in a location in the south corner of the Women's and Children's building at the Countess of Chester Hospital."

43:

"A full copy of the original and complete extraction has been provided to the defence."

That's the extraction from the entire phone. Under the sub-heading "Facebook and email":

"On the 26th and 27 June 2019, a digital forensic investigator accessed Lucy Letby's Facebook and email accounts..."

It then gives the address, [redacted]:

"... and downloaded the entire contents of the profile and messages."

My Lord, these are in italics but there's no significance in that. The times and dates of these Facebook searches placed before the jury are accurate.

So they're specifically the searches that appear in the sequence of events chart.

Section 9 entitled "Other exhibits":

"The shift rota for Lucy Letby with the exhibit reference KTL14B has been accurately compiled from the original nursing rotas obtained from the Countess of Chester Hospital for the relevant period."

Pausing there if I may, that's a document that appears at the front of jury bundle 2. If everyone would like to go to that, please, so we can remind ourselves which document that is.

I think it was left at the front. It should probably be slotted in now to divider 23. It's the coloured chart, my Lord, with shifts on. Thank you, Mr Murphy.

MR JUSTICE GOSS: So you want us to put that in section 23?

MR ASTBURY: 23, I think, which is the next available divider.

MR JUSTICE GOSS: Right, thank you.

(Pause)

MR ASTBURY: I just wonder if Mr Murphy can scroll through the remaining pages so we familiarise ourselves with the contents. We can see June and July of 2015 with the relevant colour coding. Continuing through...

(Pause)

And the last long day shift being 30 June 2016.

Thank you, Mr Murphy.

If we move, please, to agreed fact 47. Can I please distribute the document that this refers to?

(Handed)

The next divider in jury bundle 2, please, for these.

(Pause)

This was shown in the opening, but we'll look at it in a bit more detail now if we may, please. If everyone keeps it out in front of them, I'll read the admitted fact first, 47:

"The schedule entitled 'Staff presence -- temporal analysis'..."

If I ask Mr Murphy to put it up on the screen.

Thank you:

"... is an accurate record of the paediatric medical and nursing staff on duty on the NNU of the Countess of Chester Hospital at the time and dates of the events under consideration and in this trial."

If I can ask, please, everybody to look at page 1.

It suggests page 3 of 6, but there was a frontispiece, as you've often seen on other exhibits, and we didn't burden you with that.

Looking at this, everybody will see, down the left-hand side of the first column, the events which the prosecution say are significant in this case.

Pausing there, everyone will notice that for [Baby P], near the bottom of that column, there are two entries. You'll notice that the first of those entries was the event that the Crown say is significant the night before his death when he was fed and an X-ray was taken.

Along the top of the document we can see the names of the staff -- and in fact this spreads across pages 1 and 2 because there are so many staff to be considered.

In the body of the chart is a cross where the presence of a particular member of staff coincides with the particular incident on the left-hand column.

You will see from page 2 it includes not only nursing staff but doctors and indeed the consultants on the furthest right of the second page.

The Crown suggests it gives an easy representation of who was present and when. The column in light blue shading is the column for Lucy Letby.

The very bottom row gives you a tally of the number of occasions upon which any particular individual was present on the events the Crown rely upon. That's an introduction to pages 1 and 2. You'll have it in your bundle and you can consider that at the appropriate stage as and when it becomes relevant.

If we move to the third page, what's entitled chart 3, it's what's sometimes called a heat map, described here as:

"A total presence combined staff heat map."

It details each member of the staff under the heading of their job description and, very much in the same way as the row at the bottom of the previous two pages, tallies up presence for those 24 occasions. It shows in descending order of frequency the number of times each member of staff was present.

So concentrating for a moment, as the Crown would invite you to do, on the list of nurses, Lucy Letby appears on all 24 occasions. The next in the list, and there are five of them, appear on seven occasions.

There is, if it assists, in the bottom right a key to show why the colours have been chosen.

So that deals with agreed fact 47 and the associated exhibit.

MR JUSTICE GOSS: So that goes behind divider 24?

MR ASTBURY: Yes, please.

MR JUSTICE GOSS: Which is?

MR ASTBURY: Bundle 2.

MR JUSTICE GOSS: And after that I just have one more divider marked S, I think, S for spare.

MR ASTBURY: Yes. I think it may remain like that for now.

If I can move on to agreed fact 48, please:

"The video presentation of the NNU at the Countess of Chester Hospital [and we have the exhibit reference for completeness, RC20/21] was recorded on 3 October 2021."

Agreed fact 49:

"The videos of various medical procedures and equipment played for explanatory purposes have been prepared at the request of the prosecution by medical staff not involved in these proceedings."

My Lord, that covers all of the videos and presentations that we've had.

Moving on, please, to number 50, and could I ask Mr Murphy to put up J26510. The jury will remember that image, I'm sure, from the [Baby G] case. And the agreed fact reads:

"The photograph annotated by Ailsa Simpson, exhibit reference AS4, was selected from pictures of the relevant location taken by Ricky Crellin, a crime scene investigator."

If we can move on next, please, to agreed fact 51.

I'll ask Mr Murphy to put up image 25368:

"At 9.45 pm on 24 August 2020, CSI Ricky Crellin attended nursery 2 at the NNU within the Countess of Chester Hospital and took a selection of photographs. Ashleigh Hudson was present and was asked to set up the cot, room and lighting as she remembered it on 7 September 2015. She having done so, he [Ricky Crellin] took a series of six images at differing exposures. Ashleigh Hudson was asked to select which she felt best reflected the lighting on the night in question. She selected the image subsequently produced in evidence."

Which is this image, my Lord.

Moving on to agreed fact 53, please, could I ask Mr Murphy to put up page J11. Thank you.

This exhibit, CLM2, is:

"The competency assessment for administration via IV lines (exhibit reference CLM2) was obtained from, amongst other items, Lucy Letby's HR file at the Countess of Chester Hospital."

This was a document that was discussed in the parts of the interview that we heard this morning. I'm going to ask Mr Murphy to take us through page by page and perhaps enlarge it a little to see what the nature of this particular competency involved.

We see the heading:

"Assessment for safe administration of medication by bolus/intermittent administration via a long line, Broviac line or umbilical venous catheter."

We can see Lucy Letby's name on the top, various other information, including the name of the assessor, who, as she recalled in interview, was [Nurse A].

Scroll down, please, Mr Murphy. Again, everybody can look at this, it'll be on the iPads, my Lord, in due course. One can see the competencies that are required.

(Pause)

Move on, please, Mr Murphy, to the next page.

First of all, the additional boxes.

(Pause)

Could we look at the lower part, which includes the date, please, upon which this was completed?

(Pause)

Thank you. If we could move on to agreed fact 54, please, page J60, Mr Murphy, thank you.

Fact 54 reads:

"The blood transfusion workbook (exhibit reference CLM6) was obtained from, amongst other items, Lucy Letby's HR file at the Countess of Chester Hospital."

I'll ask again Mr Murphy, please, if you can take us through the document. In particular, the handwritten entries.

So references to when they are used, how they are secured and, the Crown would say, complications of having a UVC or a UAC in situ. There are four complications listed there.

MR JUSTICE GOSS: No, no, that's not strictly accurate. It says:

"Give 4 potential complications of having a UVC/UAC in situ."

And those are the four that have been written in.

MR ASTBURY: If I didn't say that, I'm sorry, that's what I meant to say. Thank you.

Then if we look at the lower part of the form, please, Mr Murphy. Reference there to spotting an air bubble in the line and what to do. Other recommendations about the position of UVC and UAC.

Continue, please. This perhaps is the type of information that can be looked at at leisure.

If we scroll through, please. Thank you. Some small handwritten entries on the form.

Thank you, Mr Murphy. We can move on in the document and look at the handwritten entries again, please.

The lower half, please. Thank you.

Further handwritten entries, or certainly tick boxes, further down the form, please.

Again, it appears to have been signed off and there's a date on the right-hand side of the form.

We can move on, please, to agreed fact 55:

"It is agreed that the handwritten notes (exhibit reference PMB8), seized from 41 Westbourne Road, Chester, on 4 July 2018 are the resuscitation notes written at the time of [Baby M]'s resuscitation."

Moving on to the next sub-heading, "Swipe data", which everyone will recall appears in some of the sequence of events charts and not others:

"On 22 April 2021, officers seized a computer base unit with an exhibit reference TTL3, which records the use of swipe fob entry data for secure access at the Countess of Chester Hospital. This data was extracted and analysed."

57:

"Insofar as the dates with which this indictment is concerned, the data was limited to periods between 12 May 2015 and 16 July 2015 and 22 October 2015 until 31 January 2018. There was no data available for the intervening period."

58:

"Officers also found individual dates within those periods when data was unavailable. However, where data was available for relevant dates, it accurately appears within the sequence of events charts."

Finally, moving on to section 10, which is headed "Lucy Letby". Fact number 59:

"Lucy Letby was born on 4 January 1990. She has no criminal convictions, cautions or reprimands recorded against her."

60:

"The NNU at the Countess of Chester Hospital was reclassified as a level 1 unit on 7 July 2016. This decision was made by the trust itself."

MR JUSTICE GOSS: Mr Astbury, can I just check? You gave some J numbers there for various documents that Mr Murphy put up on the

screen. Can I just confirm, are they on the iPad presentations or not? And if they are -- I see he's nodding.

MR ASTBURY: They're in the post-indictment section and in the additional exhibits, but at the moment I think they just appear with J numbers, so one of our housekeeping tasks is to ensure that the description matches that which has been read out, but that's going to be attended to very soon.

MR JUSTICE GOSS: Right. Well, I don't know whether the jury were making notes of those J numbers but it might help, while you have this document in front of you and it's fresh in your mind, to make a note of these J numbers in case you wish to refer to any of them in due course. So going back to agreed fact 39 on the first page of this section, section 8, telecommunications, 39. The thank-you card from the [Babies E & F] family. You've got the exhibit reference number. The J numbers are J2462 and J2463. Some of you had already written that down, I think.

41, the sympathy card to the [Baby I] family, J13163.

Down to 46. Other exhibits, section 9. The shift rota you've now got in section 23 in your second jury bundle, so I put JB23 there, just to remind you that's where it is.

Over the page, number 47. The schedule entitled "Staff presence -- temporal analysis", CEH16A, that's in JB24.

Halfway down that page, 50. The photograph by Ailsa Simpson that she selected is J26510.

51, a photograph that Ashleigh Hudson selected, J25368.

Over the page, 53, at the top, a competency assessment for administration via IV lines, CLM2, is J11 to J14. I just made the note that that was completed on 31/5/15 (sic).

54, the blood transfusion workbook, CLM6, is J60.

That's it, I think, Mr Astbury.

MR ASTBURY: Yes. Thank you, my Lord.

MR JUSTICE GOSS: Right.

MR ASTBURY: I'm reminded, that was completed 11 May 2016.

MR JUSTICE GOSS: 11 May 2016, yes. Although it was in her HR file, it doesn't actually have her name on it.

MR ASTBURY: No, that's right. It's unsigned by the subject of the training, yes.

My Lord, that concludes the prosecution case.

MR JUSTICE GOSS: Thank you very much, members of the jury.

As you anticipated, we were going to complete the prosecution evidence this afternoon, and that stage has now been reached.

Mr Myers, I think -- is the best thing just to have a short break now?

MR MYERS: Yes, my Lord, it is.

MR JUSTICE GOSS: Will 10 minutes be sufficient?

MR MYERS: Maybe we should take 20. There are a couple of matters to consider. Fifteen minutes, just in case we run over, but 10 might be a little short.

MR JUSTICE GOSS: I'll say this will be at least 15 minutes, it may be 20 minutes. The reason for this is I'm trying to make some enquiries to ascertain what happens hereafter and determining when you're going to be required again. All right? This is done in virtually every case, certainly any case of any substance at this stage, at the end of the prosecution evidence before we go any further, and it needs to be done in this case.

All right? Thank you very much. So at least 15 minutes.

(In the absence of the jury)

MR MYERS: We're grateful, my Lord, for the current rota of non-sitting days. We keep that in mind.

MR JUSTICE GOSS: Yes.

MR MYERS: Before we proceed, so far as the defence are concerned, there are two matters for the court to deal with. The first one is a matter of law, which I'll provide now to your Lordship, of course, and to the prosecution. I've indicated the general nature of that, but of course they will need time to consider that and respond.

The other matter to be considered, although having discussed this briefly with Mr Johnson, we don't anticipate it will take very long, are any particular arrangements for the court to take to assist Ms Letby with the process of giving evidence. Your Lordship's been provided with a bundle and submissions on that.

MR JUSTICE GOSS: Yes.

MR MYERS: The principal matter to be dealt with before we move to the giving of evidence, which is anticipated by Ms Letby, is the question of no case to answer. So it comes to me to serve that and for my learned friends to consider how long they would need. But it would seem to me at this stage that at the very least that will have to be considered and whatever response they see fit to make wouldn't be capable of resolution alongside the defence argument until tomorrow. So that at least would be required to deal with that matter of law.

It would seem to me it would take at least tomorrow for the court to deal with that and then, looking forwards, if I may, to assist your Lordship -- we're obviously waiting to hear what my learned friends say -- Dealing with the submission will take tomorrow and it's possible could go into Tuesday. Possible.

We don't anticipate that the arrangements concerning Ms Letby's giving evidence would add greatly to the timescale. So that would mean that, depending upon the way matters go and how your Lordship were to determine -- to deliver any ruling that follows, and of course sometimes that can be done quickly with reasons to follow later, howsoever your Lordship determines, the soonest we would come to the start of the defence case and the calling of Ms Letby to give evidence would be this coming Tuesday. That would be the soonest.

There's a possibility, if matters took longer than tomorrow to resolve, or the earliest part of Tuesday, that it may be the next available date would be when her evidence would commence, which would be Friday, 5 May.

That's possible.

I just observe this, but it may be there's little we can do about it: naturally, when considering her position and the defence case, and looking at the dates we have, if evidence -- if Ms Letby were to give evidence, which we anticipate would happen, and were that to start on Friday, 5 May, there would then follow in fact a weekend and 3 days, which is a five-day break after the first day of evidence.

Were it possible to avoid that, it seems to me that would be desirable. At the same time I recognise that we have a rather fragmented period ahead of us whatever we do. So maybe we just have to wait to see where we get to when we get there.

MR JUSTICE GOSS: Well, I've been thinking, as you would expect, about this, as we've been losing days. On the basis that the

defendant will be giving evidence, her evidence will take some time, will spread over many days, I expect.

MR MYERS: Yes, or weeks.

MR JUSTICE GOSS: Well, exactly. Many days. So I don't think that saying that we're going to have a four-day gap or a five-day gap in fact is prejudicial to anyone because the evidence is going to span a long period in any event.

MR MYERS: Yes.

MR JUSTICE GOSS: Therefore given that we are losing so many days, of necessity, I don't want to lose any more unless it is by reason of necessity. So my inclination at the moment -- I'm not saying this as a final decision but I thought it might help if I expressed my view at this stage and I will hear what Mr Johnson says -- my inclination is that as soon as we are in a position to proceed with the defence case and whatever evidence is called on behalf of the defence, then we start that, even if there's going to be a four-day gap after that.

MR MYERS: Well, I should say, with respectful agreement, that whereas ordinarily that would be something we would strive to avoid, given the inevitability of breaks in the course of Ms Letby's evidence, if that is where we go, whatever we do there are going to be breaks here and there. Therefore, we understand why your Lordship takes the view you do.

To assist as best as we can at this point, it would seem to us that it is unlikely we would start again with the jury until Tuesday.

MR JUSTICE GOSS: That's what I'm thinking at the moment.

What I'm thinking, and I will hear from Mr Johnson, but if it's anticipated that essentially tomorrow is going to be taken up with legal argument and discussion about arrangements so far as the defendant giving evidence is concerned, and I can say this to assist you, that I am understanding of the difficulties --

MR MYERS: Thank you.

MR JUSTICE GOSS: -- and there will be accommodation.

MR MYERS: We're grateful for that, my Lord.

MR JUSTICE GOSS: I'm conscious of (a) the situation of the defendant giving evidence in stressful circumstances and there will be breaks and the total period per day will not be what I consider to be excessive.

MR MYERS: We're grateful. We'll deal with that and assist the court when we come to it, but so far as we can assist right now that seems to be the way the timings are with evidence to commence on Tuesday, or possibly Friday if Tuesday is required for any further legal deliberations.

MR JUSTICE GOSS: Yes. In other words, I don't think the discussion about the arrangements will take very long at all.

MR MYERS: We don't think it will either, my Lord.

MR JUSTICE GOSS: I would have thought minutes, frankly.

MR MYERS: Certainly not as long as the size of the bundle might perhaps have otherwise led the court to believe.

MR JUSTICE GOSS: There we are. As I said, I understand what the situation is. I'll hear what Mr Johnson says.

Is there anything else you want to say, Mr Myers?

MR MYERS: Not at this stage, no.

MR JUSTICE GOSS: Mr Johnson?

MR JOHNSON: Nothing constructive to contribute, thank you, my Lord.

MR JUSTICE GOSS: I think as far as the jury is concerned, we can say they won't be required tomorrow but they should be prepared to attend, and should attend, on Tuesday --

MR JOHNSON: Yes.

MR JUSTICE GOSS: -- unless tomorrow afternoon they are informed to the contrary.

MR JOHNSON: Yes.

MR JUSTICE GOSS: The usual arrangement with which they're familiar.

MR JOHNSON: Yes. Mr Myers and I have discussed the essential basis, or at least I believe we've discussed the essential basis, of the submission and if we receive something in writing, we'll try and have something in writing with your Lordship by tomorrow morning.

Necessarily, given the limited number of hours between now and then, it won't be very long, but it may be all the better for that, because it's a fairly fundamental point.

MR JUSTICE GOSS: Well, let's wait and see. I entirely agree that that's the appropriate way of dealing with it. So that's what I'll do, then: call the jury back and say that they won't be required tomorrow but will, subject to some notification to the contrary, be required on Tuesday.

MR JOHNSON: Yes.

MR JUSTICE GOSS: Good. Thank you very much indeed.

(In the presence of the jury)

MR JUSTICE GOSS: You will recall many months ago when we first met and this case started that I said I would deal with all questions of law that arose. I've got to deal with an issue of law, I can't deal with it this afternoon, it's going to require tomorrow for it to be dealt with. So what I'm saying is that tomorrow you will not be required to attend at court. It will be the start for you of what will be a four-day weekend because it's then Saturday, Sunday and Monday is the first of the three May public holidays.

But you will be required on Tuesday to come back and continue with the trial, unless for some unexpected reason, so I'm saying that in the spirit of optimism, you are notified to the contrary tomorrow afternoon in the usual way that you are if you're told you're not required to come on the next sitting day.

You've got your list here. You know that we're coming up to a period where we are sitting intermittently, essentially, rather consecutively.

So looking at the document, Tuesday will, unless you are notified to the contrary, be a sitting day.

Then we have Wednesday and Thursday off. Friday will then be the next sitting day. Then we have another public holiday. Then we've got Monday, Tuesday and that Wednesday off, the 9th and 10th, then we're back on Thursday and so on and so forth. All right?

That's the best I can do. All right? Thank you very much for your patience, your understanding and continued diligent attention to this case and to your responsibilities as jurors in the case, which I remind you, for the umpteenth time: no communication by any means with anyone about anything to do with

this case and no research about anyone or anything to do with this case.

Tuesday of next week, please. Thank you very much.

(In the absence of the jury)

MR JOHNSON: Just for the record, my Lord, and to reflect what I understand is the agreed position, it's actually incorporated in the admissions that all the interviews are in in case anything has been edited out that's relevant or becomes relevant, I should say, and the same applies, by an understanding, as I understand it to be, about the Facebook material as well. There's an awful lot of material. There's a schedule to come, but I understand the position that as between us we are agreed that should anything arise in the course of the defendant's evidence that converts something from apparent irrelevance to relevance, then there's no issue about it being referred to.

MR MYERS: That's agreed, of course, my Lord. It goes both ways.

MR JUSTICE GOSS: Exactly. Clearly it's evidence in the case and, if required and if necessary, reference can be made to it, even though it's not directly been referred to at this stage.

MR MYERS: No, we understand.

MR JUSTICE GOSS: Whilst we've been going through those agreed facts in relation to interviews it occurred to me because when I was cross-referencing the summaries of the interviews that were given to the jury, helpfully, at the end of each baby, I realised there were slight, very slight, differences, but that's not in the least bit critical. I think it's very helpful to do it in the way it was done. If I may say so. The jury will appreciate that they've got just a small proportion of the total interviews, which would otherwise run to many volumes.

MR MYERS: Enormous, yes.

MR JUSTICE GOSS: Good.

MR MYERS: We're grateful for the work that's actually gone into them, no criticism, but they were huge to begin with.

MR JUSTICE GOSS: I know. You knew that I was encouraging as much editing as possible and it was on that understanding that they were all in evidence and it was just basically trying to put before the jury what was salient.

MR MYERS: I know certainly Mr Astbury, Mr Maher and Ms Clancy have been heavily involved in reducing them and we are both grateful to them for doing that work.

MR JUSTICE GOSS: I'm very grateful for all the work that's been done.

I'm not going to spend time now going through the proposed arrangements so far as the defendant giving evidence are concerned, I'd rather that we all got your document and started reading that at this stage. But as I've indicated, unless Mr Johnson wants to make any specific representations I'm essentially, so far as timetabling, of the mind that we should have around one-hour slots, then a more substantial than ten-minute break, so that we have essentially in the region of no more than 4 hours a day.

MR MYERS: We'd be grateful for that. We can look at the actual timing when we look at the arrangements, but we were going to ask for something along those lines, my Lord. We'll come to that.

MR JUSTICE GOSS: Exactly. All right, good --

MR MYERS: I should say the submission has now been sent.

We are putting together a bundle of documents to assist because certain transcripts are referred to, so we shall put those together and make sure your Lordship and my learned friends have those as quickly as possible, but we certainly didn't want to delay the receipt of the submission itself because it will be plain enough what we're referring to from the submission.

MR JUSTICE GOSS: That's helpful. Thank you very much.

Mr Johnson, as and when, don't worry too much about rushing your response. I'd rather that you were content that you had covered the ground you wanted to cover, not necessarily in as much detail as you may want, but at least address the points you want to make, whatever they may be.

Mr Myers?

MR MYERS: Yes, we would be grateful if we could see Ms Letby now.

MR JUSTICE GOSS: Thank you. The court will sit at 10.30 tomorrow then.

(3.43 pm)

(The court adjourned until 10.30 am on Friday, 28 April 2023)